# ADMISSION RECORD

ankura hospital for women & children | 9M by ankura hospital Birth & Beyond

## Patient Movement Record

| Patient Movement (Shifting) information | | | | | OP/ER/Special Room/NICU/PICU/OT | | |
|---|---|---|---|---|---|---|---|
| S.No | Date | Time | Shifted From | Handed over by -Staff Name & Sign | Shifted to | Taken over by -Staff Name & Sign | Remarks, if any |
| 1 | 27/6 | 9.03 pm | ER | NICU | | Roja | |
| | | | | | | | |
| | | | | | | | |
| | | | | | | | |
| | | | | | | | |
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| | | | | | | | |

# INITIAL ASSESSMENT

(Done By Nurse)

| | | | |
|---|---|---|---|
| Patient Name: B/O Vagadevi | | MR.NO: 608635 | IP.NO: LB26/3007 |
| GA: wks | Birth Weight: grams | Date/Time of assessment: | |
| TOD: | Mother Blood Group: | | |

**Reason for admission/referral:**

**VITAL SIGNS:**

| | | |
|---|---|---|
| Temperature: 36.2 | HR: minutes | RR: minutes |
| Retractions: nil | SAS: | Pulse Volume: Good |
| B.P: 58/38 (45) | Spo2: 95% | GRBS: 36mg/dl |

Weight: 2.9 kg Kg/gms, Length: 45 cms, HC: 33 cms

**PAIN ASSESSMENT:**

| Process | Indicator | 0 | 1 | 2 | 3 | Score |
|---|---|---|---|---|---|---|
| Chart | Gestational Age | 36 weeks or more | 32-35 weeks 6 days | 28-31 weeks 6 days | Less than 28 weeks | 0 |
| Obser vation | Behavioural State | Active, awake, eyes open, facial movement | Quite, awake, eyes open,no facial movement | Active, sleep, eyes closed, no facial movement | Quite, sleep, eyes closed, no facial movement | 0 |
| Observe baseline heart rate & saturations for 30 seconds | Heart Rate Maximum | 0-beats per minute increase | 5-15 beats per minute increase | 15-24 beats per minute increase | 25 beats per minute increase | 0 |
| | Oxygen Saturation minimum | 92 – 100 % | 89 – 91 % | 88 – 85 % | < 85 % | 01 |
| Observe infant's facial action for 30 seconds | Brown Bulge | None | Minimum | Moderate | Maximum | 0 |
| | Eye Squeeze | None | Minimum | Moderate | Maximum | 0 |
| | Naso-labial furrow | None | Minimum | Moderate | Maximum | TS= 01 |

**INTERVENTION:**

Name: Roja

Date: 27/6/26

Sign:

Time: 9:15pm

# INITIAL ASSESSMENT
(Done By Doctors)

ankura hospital for women & children | 9M

| | |
|---|---|
| Patient Name: B/O vagdevi | MR No.: |
| IP No: | Date & Time of Assessment : 27/6/26 |
| DOB/Gender: 23/6/26, male | Assessment Done By: DR-Sowmya |

**PAST OBSTETRIC HISTORY** G2P1L1

| G | GA | B.W (gms) | Sex | Age | Significant details |
|---|---|---|---|---|---|
| P1L1 | - | 3/5yrs | | | LSCS |

**OBSTETRIC HISTORY**

ANC Registered/Non Registered

LMP EDD T1 EDD

Mother's Blood Group A+ve Last Hb _____

HIV _____ HBsAG _____ VDRL _____ TORCH _____

USG: Polyhydramnios/Normal/ Oligohydramnios

IUGR : Yes/No

UA Flow: Normal/Diminished /Absent/ Reverse

Antenatal Steroid: Drug _____ Dose _____

Time-lapse since last dose:

Magnesium Sulphate for Neuro Protection: Yes/ No

**MATERIAL RISK FACTORS**

| | |
|---|---|
| Consanguinity | Yes/ No |
| Age <18/ >35 yrs | 40yrs Yes/ No |
| PIH/Eclampsia | MC-10yrs Yes/ No |
| GDM/NIDDM/IDDM | Yes/ No |
| PROM >18hrs/ Pre PROM | Yes/ No |
| APH | Yes/ No |
| Maternal Fever/ Chorioamnionitis | Yes/ No |
| IHD Asthma | Yes/ No |

Chronic Illness: _____

Medication: _____

Other Problems, if any Previous Sibling Death:

Preterm Labour: Yes/ No

Assisted Reproduction: Yes/ No

**PERI-PARTUM HISTORY** Birth Weight: 2.1kg

Date of Delivery 23/6/26 Time 11:31PM

Fetal distress Yes/No

Mode of Delivery: SVD/ Forceps/Ventouse/LSCS

If LSCS: ☑ Emergency ☐ Elective

Indication for LSCS: previous LSCS in labour

Presentation: Vertex/ Breech Liquor clear/

Meconium/ Foul smelling

Resuscitation: cried at birth/ stimulation/ Bag

Mask/ Intubation/ chest compressions/ adrenaline

APGAR: 1 Min _____ / 5 Min _____ / 10 Min _____ NK

DCC: Yes/ No _____ Seconds

Cord ABG:

# PHYSICAL EXAMINATION

ankura hospital for women & children | 9M

## HEAD - NECK

| | |
|---|---|
| | **Normal** |
| | Microcephaly |
| | Macrocephaly |
| | Moulding |
| | Caput |
| | Cephalhematoma |
| | Abnormal Sutures |
| | Abnormal Fontanelles |
| | Scalp Electrode Mark |
| | Scalp |
| | Abscess |
| | Abnormal Facies |
| | Cleft Lip: ☐ Unilateral |
| | Cleft Lip: ☐ Bilateral |
| | Cleft Palate |
| | Nasal Flaring |
| | Neck Masses |
| | Others: |

## EARS

| | |
|---|---|
| | **Normal** |
| | Abnormal Location |
| | Abnormal Form |
| | Preauricular Sinus (pit) |
| | Preauricular Skin Tags |
| | Others: |

## EYES

| | |
|---|---|
| | **Normal** |
| | Abnormal |

## CHEST - LUNG

| | |
|---|---|
| | **Normal** |
| | Abnormal |
| | Accessory Nipples |
| | Abnormal Chest Shape |
| | Grunting |
| | Retractions |
| | Tachypnea |
| | Apnea |
| | Diminished Breath Sounds ☐R ☐L |
| | Stridor |
| | Others: |

## CARDIOVASCULAR

| | |
|---|---|
| | **Normal** |
| | Murmur |
| | Dysrhythmias |
| | Tachycardia (>160/min) |
| | Bradycardia (<100/min) |
| | Decreased Precordial Activity |
| | Four Limb Pulses |
| | Others: |

## ABDOMEN

| | |
|---|---|
| | **Normal** |
| | Distended Abdomen |
| | Scaphoid Abdomen |
| | Palpable Mass (Location) |
| | Hepatomegaly |
| | Splenomegaly |
| | Umbilical Cord |
| | Genitalia |
| | Anus |
| | Others: |

## MUSCULOSKELETAL

| | |
|---|---|
| | **Spine:** Normal |
| | Abnormal |
| | Specify: |
| | Limbs: |
| | Fracture: |
| | Specify: |
| | DDH (Developmental |
| | Dysplasia of Hip) |
| | Malformation (Specify): |
| | Others: |

## NEUROLOGY

| | |
|---|---|
| | **Normal** |
| | Hypotonia |
| | Hypertonia |
| | Abnormal Cry |
| | Abnormal Moro: ☐R ☐L |
| | Asymmetric Movement |
| | Paralysis |
| | Suck: ☐Present ☐Absent |
| | Gag: ☐Present ☐Absent |
| | **Others:** |

## SKIN

| | |
|---|---|
| | **Normal** |
| | Acrocyanosis |
| | Central Cyanosis |
| | Pallor |
| | Plethora |
| | Bruising (Location) |
| | Laceration: |
| | Petechiae |
| | Jaundice |
| | Meconium Staining |
| | Edema (Specify): |
| | Birthmark (Specify): |
| | Others: |

# PHYSICAL EXAMINATION

ankura hospital for women & children | 9M by ankura hospital Birth & Beyond

| | | | |
|---|---|---|---|
| **Feeding History** | | **Breast Fed / Top Fed:** | |
| **Gestational Age by Dates:** _____(weeks) by $T_1$ US: _____(weeks) by New Ballard Score : _____(weeks) | | | |
| **Anthoropometry:** | | | |
| **Length** | cm | **Head Circumference** | cm |
| **Admission Weight** | kg | **Chest Circumference** | cm |

**VITAL SIGNS:**

**Heart Rate:** 130 /Min **RR:** 46/min. **Blood Pressure:** _____ mm Hg

**Temperature:** 98.6°F **$SpO_2$:** 98% on **on $FiO_2$** **Pain Score:** _____ nasal prongs

**PROVISIONAL DIAGNOSIS:**

Late preterm 36 wk | SGA 2.1kg | male | ENCEPH | CHB | Hypoglycemia Seizures.

GRBS: 82mg/dl

**INVESTIGATIONS :**

B/G/T
CBP
CRP
RFT
VBG
Blood cls
LFT
RBS

**TREATMENT PLAN**

- IV fluids (GIR:)
- Inf NS bolus
- Inj. TAXIM

**Name of the Doctor:** Dr. Sravya **Date :**

**Signature of the Doctor:** **Time:**

# NURSE'S HAND OFF COMMUNICATION (SBAR)

ankura hospital for women & children | 9M by ankura hospital

Patient Name: B/O Vagadevi Age/Sex: male MR No./IP No.: 608635

DOA: 27/6/26 Diagnosis: Seizures Surgery/Procedure:

POD: Food/Drug Allergies: Past Health History:

| | | | | | |
|---|---|---|---|---|---|
| SITUATION & BACKGROUND | | Area | | | NICU |
| | | Shift | | | 8pm-8AM |
| | | Diet - Therapeutic Diet | | | NPO till further |
| | | Dependent/Independent in feeding | | | dependent |
| | LINES | | | | IV Cannula |
| | | | | | |
| | | | | | |
| ASSESSMENT | | Infusion / Transfusions | | | 10%+ SD |
| | | GCS/LOC | | | — |
| | | Pain score | | | -2- |
| | | DVT prophylaxis | | | |
| | | VITALS | | | HR within specific |
| | | Ventilation | | | Room air |
| | | Bowel movement/flatulence | | | ✓ |
| | | Void/foley | | | Void |
| | | Genital discharge | | | Normal |
| | | Skin integrity/Score | | | — |
| | | Fall risk/Score | | | - |
| | | Restrains | | | - |
| | | ADL | | | — |
| RECOMMENDATIONS | PROCEDURES & INVESTIGATIONS | Critical lab values | | | — |
| | | Ordered / planned | | | ECG, NSG |
| | | Reporting pending | | | Blood cg |
| | | Plan of care | | | Warm Care |
| | REFFERAL | Consultations | Time of inform / Time of visit | Time of inform / Time of visit | Time of inform / Time of visit |
| | | DMO Orders | | | |
| | | DMO sign | | | |
| | | Remarks | | | |
| | | Date/Time | | | 27/6/26 8pm-8AM |
| | | Handing over given by Name / ID | | | Loje 10636 |
| | | Taken over by Name / ID | | | |

# NURSE'S HAND OFF COMMUNICATION (SBAR)

ankura hospital for women & children

9M by ankura hospital

28/6/26

Patient Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Na _____ Age/Sex: _____ MR No./IP No.: _____

DOA: _____ Surgery/Procedure: _____

POD: _____ s: _____ Past Health History: _____

| | | | | | |
|---|---|---|---|---|---|
| SITUATION & BACKGROUND | | Area | NICU | NICU | NICU |
| | | Shift | morng | Evening | 8pm - 8Am |
| | | Diet - Therapeutic Diet | NPO | 10ml 2nd hrly | 10 ml 2ndly. |
| | | Dependent/Independent in feeding | Dependent | TF - EBM | TF-EBM |
| | LINES | | IV cannula | UVC | UVC |
| | | | | IV cannula | IV Cannula |
| ASSESSMENT | | Infusion / Transfusions | IV. Fluids | 10% DP D5 | 10% D |
| | | GCS/LOC | Normal | Conscious | Conscious |
| | | Pain score | —01— | -01- | -02- |
| | | DVT prophylaxis | — | — | — |
| | | VITALS | HR.106 SpO2.100 | HR: 112, SpO2:96% | HR 115 bpm SpO2 98% |
| | | Ventilation | NPO2 | RIA | O2 Support |
| | | Bowel movement/flatulence | Not passed | Not passed | |
| | | Void/foley | Foley | Foley | Foley |
| | | Genital discharge | NO | X | |
| | | Skin integrity/Score | Normal | DSG | Normal |
| | | Fall risk/Score | —0— | -0- | — 0 — |
| | | Restrains | Side rails | — | — |
| | | ADL | | — | — |
| RECOMMENDATIONS | PROCEDURES & INVESTIGATIONS | Critical lab values | | | — |
| | | | | — | — |
| | | | | | — |
| | | Ordered / planned | UVC · feeds | | UVC line. |
| | | | EEG. NSG | — | NSG D day |
| | | Reporting pending | | BID CLS | Blood C/S |
| | | | Blood culture | EEG | |
| | | | EEG. | NSG (P) | — |
| | | Plan of care | Warm care | | |
| | | | feeds | CST | Warm Care. |
| | REFFERAL | Consultations | Time of inform / Time of visit | Time of inform / Time of visit | Time of inform / Time of visit |
| | | | | | |
| | | DMO Orders | | | |
| | | DMO sign | | | |
| | | Remarks | | | |
| | | Date/Time | 28/6/26 8am-2pm | 28/06 2pm-8pm | 28/6 8pm-8AM |
| | | Handing over given by Name / ID | Prasuja. 12272 | Shanty - 2654. | Raji 10536 |
| | | Taken over by Name / ID | Shanty | | |

# NURSE'S HAND OFF COMMUNICATION (SBAR)

29/6/26

ankura hospital for women & children | 9M

Patient Name: B/o Vagdevi Age/Sex: _____ MR No./IP No.: _____

DOA: 27/6/26 Diagnosis: Seizures Surgery/Procedure: _____

POD: _____ Food/Drug Allergies: _____ Past Health History: _____

| | | | | | |
|---|---|---|---|---|---|
| SITUATION & BACKGROUND | | Area | NICU | NICU | NICU |
| | | Shift | morning | Evening | 8pm-8AM |
| | | Diet - Therapeutic Diet | 15ml 2nd hrly | 15ml andby/ | 15ml/2nd |
| | | Dependent/Independent in feeding | s/f- EBM | B/F EBM | s/f EBM |
| | | | | — | — |
| | LINES | | UVC | UVC | UVC |
| | | | IV cannula | RV cannula | IV Cannula |
| | | | | — | - |
| ASSESSMENT | | Infusion / Transfusions | 10%.D-10.4ml/hr | 10%.D-Dextrose | 10%. D-10.4 |
| | | GCS/LOC | Conscious | Conscious | Conscious |
| | | Pain score | -01- | -01- | -01- |
| | | DVT prophylaxis | — | | |
| | | VITALS | HR:128, SpO2:96% | HR-146 SpO2 96 | 142/mt Spo2 96% |
| | | Ventilation | RIA | RIA | Room air |
| | | Bowel movement/flatulence | ✓ | ✓ | ✓ |
| | | Void/foley | ✓ | ✓ | Void |
| | | Genital discharge | X | X | X |
| | | Skin integrity/Score | Normal | Normal | Normal |
| | | Fall risk/Score | — | — | — |
| | | Restrains | — | — | — |
| | | ADL | — | — | — |
| RECOMMENDATIONS | PROCEDURES & INVESTIGATIONS | Critical lab values | — | — | — |
| | | Ordered / planned | NSG | MRI TIm | — |
| | | | Neurologist opinion | | — |
| | | Reporting pending | — | — | — |
| | | Plan of care | CBT | Continue Same | Warm Care |
| | REFFERAL | Consultations | Time of inform / Time of visit | Time of inform / Time of visit | Time of inform / Time of visit |
| | | DMO Orders | | | |
| | | DMO sign | | | |
| | | Remarks | | | |
| | | Date/Time | 29/6/26 8Am-2pm | 29/6/26 2pm to 8pm | 29/6/26 8pm-8AM |
| | | Handing over given by Name / ID | Sr. Raja | Sandhya | Raja C0636 |
| | | Taken over by Name / ID | Sr. Shanky | | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# NURSE'S HAND OFF COMMUNICATION (SBAR)

30/6/26

ankura hospital for women & children | 9M by ankura hospital

Patient Name: B/O Vaydevi Age/Sex: male MR No./IP No.: 608635

DOA: 27/6/26 Diagnosis: Seizures Surgery/Procedure:

POD: Food/Drug Allergies: Past Health History:

| | | | | | |
|---|---|---|---|---|---|
| SITUATION & BACKGROUND | | Area | NICU | NICU | NICU |
| | | Shift | morning | Evening | 8pm-8am |
| | | Diet - Therapeutic Diet | dependent | 30ml 2nd hrly | S/F 2nd hrly |
| | | Dependent/Independent in feeding | spoon fed | SIF - FF | Dependent |
| | LINES | | NO | | |
| | | | IV | UVC | UVC |
| | | | - | | |
| | | | UVC | | |
| ASSESSMENT | | Infusion / Transfusions | Stop | 10%.D-3.5ml/hr | 10%.D-3.5ml/hr |
| | | GCS/LOC | "0" | Conscious | Conscious |
| | | Pain score | "0" | -02- | -02- |
| | | DVT prophylaxis | - | - | - |
| | | VITALS | HR:146·Spo-96 | HR: 128, Spo: 96% | HR:142 Rt Spo:96% |
| | | Ventilation | - | - | - |
| | | | Roomair | RIA | Roomair |
| | | | - | | |
| | | Bowel movement/flatulence | ✓ | ✓ | ✓ |
| | | Void/foley | - | ✓ | ✓ |
| | | Genital discharge | "0" | X | X |
| | | Skin integrity/Score | - | Normal | Normal |
| | | Fall risk/Score | - | - | - |
| | | Restrains | NO | - | - |
| | | ADL | - | - | - |
| RECOMMENDATIONS | PROCEDURES & INVESTIGATIONS | Critical lab values | NO | | - |
| | | | - | - | - |
| | | | - | | - |
| | | Ordered / planned | MRI Today | - | MRI Today |
| | | Reporting pending | | - | - |
| | | Plan of care | Continue room | - | Continue Warm |
| | REFFERAL | Consultations | Time of inform / Time of visit | Time of inform / Time of visit | Time of inform / Time of visit |
| | | | | | |
| | | DMO Orders | | | |
| | | DMO sign | | | |
| | | Remarks | | | |
| | | Date/Time | 30/6/26 8pm to 2pm | 30/6/26 2pm-8pm | 30/6/26 8pm-8am |
| | | Handing over given by Name / ID | Geetha 12416 | Shanty -2634. | Rafa 10636 |
| | | Taken over by Name / ID | Shanty -2634 | | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 27/6/26 | A 5 day old male baby delivered by EMLCS (Previous inlabour Late preterm, at Area hospital Sangapet. Baby CIAB, shifted to mother side at 2HOL & on exclusive breast feeding.<br>On DOL 4, baby had generalised tonic clonic fits 4 focal seizures 4 sent to private hospital when Inj. Levipil loading dose given. Sugar was checked<br>Fits ↓ 10 mg/kg bolus<br>3 mg/kg/hr Isolyte P<br>↓<br>2 mg/kg/hr Isolyte P<br>Baby was advised to admit IP/O hypoglycemia & seizures but parents brought here for further management | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|

At the time of admission baby cries on warm prongs, Temp: 98.6°F GRBS: 82mg/dl dec activity + SIO dehydration +

LOC: 2.15

wt: 2.280kg Baby once shifted to PICU started 1kg on GIR 8mg/kg/min. After sending samples Inj TAXIM given.

VBG
pH: 7.373
pCO2: 37.3
HCO3: 21.3
BE: -4
Lac: 2.15
BP: 74/38

Baby had seizure generalised seizures. Inj. Lorpal started on Inj. Phenobarbitone loading dose

82 7.36 GRBS: 66mg/dl

45 1 hr 77mg/dl

2:00 75

102 mg/dl

Inj. Pyridoxine
Inj. VIT-K

CBP: 2.4 mg/dl S.Ca: 8.6
S.Creat: 0.4 Na/K/Cl: 132/5.1/102
BU: 20 PCV: 49.8
TC: 9100 (N: 68%) PLT: 1,83,000

Bone

# PROGRESS NOTES

ankura hospital for women & children | 9M

Patient Name: B/o Vagadevi MR No.: 162608635

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| | Date :- 28/6/26<br>DOL :- D4 day<br>Intake :- Npo Rta fluid<br>Output :- 30ml [1.03ml/kg]<br>GRBS :- 105mg/dl<br>NIBP :- 58/38 45<br>Stool :- Stool not passed<br>Abs :- Inj. Tonicon D2 | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 28/6/26<br>9 A.M | C/S/B kaarthikeya s/o. | |

multiple seizure episodes (+)
Baby on minimal oxygen 0.1 litres/min
GRBS – 105 mg/dl
Activity better
AF flat
Perfusion (N)
Pulses – WF
Dehydration (+)
HR – 127, $SpO_2$ – 99%
B.P – 64/43(50) mm Hg
U.O – 1.8 ml/kg/hr

NSG
EEG
BNBS

Plan

1) TF 10 ml 2nd hrly.
2) LFNC 0.1 litres/min
3) SOS ventilation
4) Levipill/Phenobarbit maintenance
5) Inj. Optineuron x 3days
6) TV – 150 ml/kg/day
GIR – 8

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 28/6/26 | Seen orders<br>① LFNC 0.1 lt/min<br>② TV – 150 ml/kg/day<br>GIR – 8<br>③ T/F 10 ml Q 2H<br>④ Inj. Lipid 10mg/kg/dose – BD<br>⑤ Inj. Phenobarbitone 5mg/kg/day<br>⑥ Inj. Optineuron D2/3<br>⑦ ventilation – SIMV D2 ⑩ Inj. Tonim<br>⑧ TO do NSG ⑪ IV cal gluconate Q 6 Hrly<br>EEG ✓<br>ENBS<br>⑨ Fosphenytoin – SOS | |

# PROGRESS NOTES

ankura hospital for women & children | 9M by ankura hospital

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 29/6/26<br>8 am | Late preterm / 36 weeks / SGA / 2.1 kg /<br>Em. LSCS<br>/<br>prev LSCS in labour<br>/ Hypoglycemic seizures<br><br>On Room air since 6 am<br>SpO2 – 98%.<br><br>Last seizure episode 5 am on 28/06/26<br>On Lenipil IV maintenance (20mg/kg/day)<br>Gradenal IV maintenance (5mg/kg/day)<br>D3 Optineuron<br>on T/F 10ml Q2H + IVF 10%.D @ GIR 8mg/kg/min<br>GRBS – T5 → 105 → 93 → 50 → (@ GIR -6)<br>D3 Cepotaxime<br>IV Calcium 8mg/kg/day | |

# PROGRESS NOTES

ankura hospital for women & children | 9M by ankura hospital

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| | EEG – Diffuse cerebral dysfunction<br>no seizure activity.<br><br>Plan MRI brain<br>Neurologist opinion today.<br><br>PH – 7.37<br>$PCO_2$ – 37.3<br>$HCO_3$ – 21.3<br>Lactate – 2.15<br>TLC – 9100<br>Plt – 1.83 L<br>PCV – 49.8<br><br>CRP – 2.4<br>$Ca^{++}$ – 8.6<br>Creat – 0.4<br>Urea – 20<br>Electrolyte – 142 / 5.1 / 107<br>Ammonia – 86<br>TSB – 15.5 | |

# PROGRESS NOTES

ankura hospital for women & children

9M by ankura hospital

Patient Name: Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| | Date :- 29/6/26<br>DOL :- D6 day<br>Intake :- H/R 104 2ndly<br>Output :- urine passed<br>URBS :- 93 mg/dl<br>T.wt :- 2.24 kg<br>Y wt :- 2.2 kg [40 gms ↑]<br>N1 BP :- 65/44 (51)<br>Abx :- Inj ± Tonacef (D3)<br>Stool :- 1 time passed | |

# PROGRESS NOTES

ankura hospital for women & children | 9M

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| | DY. S-mesko sis<br>36 wks \| DOL- 7 days \| seizures<br>Hypoglycemia,<br>No RD,<br>Sensorium improve<br>No focal s<br>Shoulder<br>AF level<br>No dismorphism<br>Screen Negative.<br>No seizures after admissity<br>Neurology opinion. EELAK Normal.<br>MRI awaited<br><br>Tab Cefim CALT<br><br>plan<br>SF + DBF 15-20ml<br>fluid CALT<br>Stop: optineuron<br>Stop: Levipil<br>Oral Calcium plus 2.5 mL TID. | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 29/8/26<br>9 AM | OB Dr. Kaushik sir<br>36 wks / 2.1 kg / Hypoglycemic seizures<br>on room air<br>NO seizures ~~last~~ last 24 hrs<br>on GIR - 8<br>+ T/F 10ml Q 2H<br>NO Hypoglycemia<br>MRI Brain ~~office~~ Neurologist opinion<br>① Tcain tidal Catheter<br>② Syp. Caffeine<br>Plan<br>① Neurologist opinion)<br>② T/F 15 ml Q 2H<br>③ Cont Phenobarbitone<br>④ Stop Levipil<br>⑤ Optineuron x (5) day | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 29/6/26<br>11 A.M | fresh orders<br>1) S/F 15 ml 2nd baby<br>2) IVF 10% D $\bar{c}$ G1R-8 @ 10.5 ml/hr<br>3) Stop Inj. Levipill<br>4) Inj. Phenobarbitone maintenance dose<br>D2/D3 5) Inj. Optineuron<br>6) Inj. Taxim till cultures<br>7) Neurologist opinion<br>8) MRI Brain<br>9) Oral Calcimax plus<br>10) Warmth Care<br>11) Monitor Vitals<br>12) GRBS monitoring 6th baby 6h | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 29/6/26 | C/S/B Sreeram sir<br>hypoglycemia w/ IUGR<br>Baby on room air<br>Stable<br>last seizure 36 hours back<br>sensorium improving<br>GE & AF – normal<br>UL – mild hypotonia, head lag (+)<br>LL – (N) Tone<br>hydration – (N)<br>AF level<br>Abn soft<br>Tolerating feeds<br><br>Plan<br>1) S/F adlib<br>2) Inj. Taxim<br>3) IVF 10% D GIR-8<br>4) Critical sample if GRBS < 50 mg/dl<br>5) | |

Sr. Cortisol Blood before ENBS

# PROGRESS NOTES

ankura hospital for women & children | 9M

Patient Name: _____ MR No.: _____

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| | Date :- 30/6/26<br>DOL :- D7day<br>Intake :- S/cadres.<br>Output :- urine/stool passed<br>T.wt :- 2.30kg<br>y.wt :- 2.24kg<br>alBP :- 58/38 mm<br>GRBS :- 109mg/dl<br>ABS :- Inj. Taxim (D4)<br>Stool :- 3 time passed | |

# PROGRESS NOTES

ankura hospital for women & children | 9M by ankura hospital

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 29/6/26<br>3:30PM | S/B Dr. Keerthi (Pediatrics)<br>Recurrent hypoglycemia - Non ketotic<br>Acute symptomatic seizures.<br>JUGR \| SGA.<br>1 No acidosis<br>Normal ammonia<br>Non ketotic<br>O/E: Af @ cvet<br>no cataracts<br>No organomegaly<br>excessive Body hair (+).<br>Adv<br>- To send S. Insulin<br>S. cortisol<br>Blood ketones ] in next critical sample.<br>- MRI Brain (Plain) once sugars are Stabilized. | |

# PROGRESS NOTES

ankura hospital for women & children | 9M

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 30/6/26<br>9 A.M | DOL-7 / Late Preterm 36 weeks / EM-LSCS i/v/o Prev-LSCS in Labour / Symptomatic hypoglycemia / Neonatal seizures<br><br>Symptomatic hypoglycemia<br><br>Baby seizure free x 48 hours<br>on inj. phenobarbitone maintenance dose<br><br>2.30kg<br>↑ 60gm<br><br>Last GRBS – 109 mg/dl<br>on spoon feeds adlib & extra IVF with G1R–6<br>maintaining sugars > 60 mg/dl<br><br>Plan<br>MRI Brain<br>ENBS<br>Urine culture<br><br>CUE – Bacteria (+)<br>on Inj. Cefotaxim D3 | CIAB |

# PROGRESS NOTES

ankura hospital for women & children | 9M by ankura hospital

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 30/06/26<br>9:15 am | S/B Dr. Srinivas Murki<br>Late preterm / 36 weeks / DOL – 7 / 2.11 kg<br>Hypoglycaemic seizures / Severe / IVGR<br>prog improved<br>pulses w/F<br>No seizures x 48 hrs<br>AF → ↓ Gross head lag<br>Scarf crossing midline<br>Shoulder hypotonia<br>Spine (N)<br>LE Tone (N)<br>Sensorium good ; activity poor<br>$SpO_2$ – normal<br>No focal deficits<br>No hypoglycemia last 24 hrs<br>MRI brain today<br>Plan →<br>– Taper IV fluids 1ml q6H<br>– S/F ad lib<br>– ct pyridoxine till MRI report<br>– ct phenobarb<br>– Ab x for 5 days<br>Dr. Srinivas Murki<br>M.D,(Ped),DNB.,DM(Neonatology)<br>Reg. No. 50515<br>Consultant Neonatologist | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 30/6/26<br>10am | Fresh Orders<br>1) S/F ad lib 30ml Q2H<br>2) Taper and stop IV fluids 1ml Q6H<br>3) inj. phenobarbitone maintenance<br>4) ct T. pyridoxine till MRI report<br>5) inj. Taxim (D4/5)<br>6) Syp Calcimax plus<br>7) GRBS monitoring Q6H<br>8) MRI brain<br>9) Monitor vitals | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 30/6/24<br>6 PM | UB Dr. Seen on<br>On seizure on<br>No ~~further~~ seizures<br>No Hypoglycemic episodes<br>Post MRI Brain<br>Hypotonia (+)<br>~~vitals~~ - stable<br>~~Accepting~~ oral ~~feeds~~<br>Plan<br>D4/5 TKVIM<br>↓<br>Stop<br>Stop Pyridoxine | |

# PROGRESS NOTES

ankura hospital for women & children | 9M

Patient Name: B/o Vagdevi MR No.: 162608635

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| | Date :- 1/7/26<br>DOL :- Decdy<br>Intake :- S/F adlib<br>Output :- Urine passed<br>T. wt :- 2.26 kg<br>Y. wt :- 2.30 kg<br>ABS :- Dy : Tonsion Ds-<br>Stool :- 4 time passed<br>GRBS :- 94 mg dl | |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 01/07/2026<br>8am | DOL-8 \| Late Preterm 36 weeks \| E M-LSCS i/v/o Preev-LSCS in Labour \| CIAB<br>Symptomatic hypoglycemia / Neonatal seizures<br>On Room air ; NO RD<br>SpO2 – 96%<br>HR – 120/mt<br>NO further seizures x 72 hours<br>– on Syp Levodenal<br>Hypoglycemia :<br>109 → 108 → 99 → 94<br>IV fluids stopped ; on only feeds<br>Nutrition :–<br>on S/F 30ml 2HT<br>Current wt – 2.26 kg (40g ↓)<br>V/O – adequate<br>Blood c/s – negative – y'day events<br>MRI – s/o hypoglycemic brain injury | Stopped Taxim- |

# PROGRESS NOTES

Patient Name: _____ MR No.: _____

| DATE & TIME | PROGRESS NOTES<br>PAIN SCORE: ____ (REQUIRED INTERVENTION IF PAIN SCORE IS ≥ 4) | NAME & SIGN |
|---|---|---|
| 1/7/26<br>9 AM | SJB Dr. Kaushik w<br>On room air<br>NO ~~fever~~ reported/<br>NO Hypoglycemic episodes x 48hrs<br>HR - 113/min, SPO2 - 94%<br>Post MRI - on ~~feed~~ T/F OA<br>on syp. Gabapentin<br>~~Activity~~ improved<br>UL & Shoulder - Hypotonia<br>Plan<br>① S/F ② OH<br>③ syp. Gabapentin<br>④ syp. Calcirol pulse<br>⑤ Neurodev F/U<br>⑥ Remove line<br>BERA<br>Shift to room | |

# MEDICATION CHART

ankura hospital for women & children

9M by ankura hospital Birth & beyond

Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ Weight: _____ Height: _____

Age/ Gender: _____ Ward / Room No: _____

Allergies & Adverse drug reactions: ☐ Yes ☐ No

If yes: Medicine( or other), Reaction/ type/ Date

(Sign, Date & Time )

## STAT Medications / Anesthesia Pre-Medications / SOS

| Date & time (Prescribed) | Medicines | Dose | Route | Frequency | Prescribed by Name & Sign | Date & Time of Administration | Nursing Staff Name & Sign Given | Check |
|---|---|---|---|---|---|---|---|---|
| 27/6 | IVF NS 5dns | | | stat | | 27/6 | | |
| 27/6 | Inj. LEVIPIL | 45 mg | IV | stat | | 27/6 | | |
| 27/6 | Inj PHENOBARBITONE | 45 mg | IV | stat | | 27/6 | | |
| 27/6 | Inj PHENOBARBITONE | 22 mg | IV | stat | | 28/6 | | |
| 27/6 | Inj. Vit K | 1mg | IV | stat | | 28/6 | | |
| 27/6 | Inj. OPTINEURON | | IV | stat | | 28/6 | | |
| | | | | | | | | |

## Verbal/Telephone orders (to be signed within 24 hrs. of order)

| Date & time (Prescribed) | Medicines | Dose | Route | Frequency | Prescribed by Name & Sign | Date & Time of Administration | Nursing Staff Name & Sign Given | Check |
|---|---|---|---|---|---|---|---|---|
| | | | | | | | | |
| | | | | | | | | |
| | | | | | | | | |
| | | | | | | | | |
| | | | | | | | | |
| | | | | | | | | |
| | | | | | | | | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# MEDICATION CHART

ankura hospital for women & children | 9M by ankura hospital

**Regular Medication** — IP No: LB/26/3007

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 27/6/26 | INJ. TAXIM | 105mg | I.V | 12th hrly | | |

| Date | 27/6 | | 28/6 | | 29/6 | | 30/6 | | | |
|---|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | 10 AM | x | 10 AM | Anuf | 10 AM | Shancy | 10 AM | Sujitha | | T dy |
| TIME GIVEN | | | | | | | | | | stop |
| TIME GIVEN | 10 PM | Reje | 10 PM | Reje | 10 PM | Reje | 10 PM | | | |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign | |

**Regular Medication** — IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 28/6/26 | INJ. CALCIUM GLUCONATE | 4.5 ml | I.V | 6th hrly | 4.5ml + 4.5ml | |

| Date | 28/6/26 | | 29/6 | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | 6 AM | x | 6 AM | Reje | | | | | | S |
| TIME GIVEN | 12 PM | Anuf | 12 PM | | | | | | | T |
| TIME GIVEN | 6 PM | Shy | 6 PM | | | | | | | O |
| TIME GIVEN | 12 AM | Reje | 12 AM | | | | | | | P |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign | |

**Regular Medication** — IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 28/6/26 | INJ. PHENOBARBITONE | 5.5mg | I.V | 12th hrly | | |

| Date | 28/6/26 | | 29/6/26 | | 30/6 | | 1/6 | | | |
|---|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | | | | | | | 6 AM | Reje | | change to oral |
| TIME GIVEN | 6 PM | Shy | 6 AM | Reje | 6 AM | Reje | | | | |
| TIME GIVEN | | | | | | | | | | |
| TIME GIVEN | | | 6 PM | | 6 PM | Shancy | 6 PM | | | |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign | |

**Regular Medication** — IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 28/6 | inj. LEVIPIL | 25mg | IV | 12th hourly | | |

| Date | 28/6/26 | | 29/6/26 | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | | | | | | | | | | S |
| TIME GIVEN | 12 PM | Anuf | 12 AM | Reje | | | | | | T |
| TIME GIVEN | | | | | | | | | | O |
| TIME GIVEN | | | 12 PM | | | | | | | P |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# MEDICATION CHART

ankura hospital for women & children | 9M by ankura hospital

## Regular Medication

IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 28/6/26 | Inj. OPTINEURON | 3 ml | IV | OD | X 3 days | |

| Date | 28/6 | | 29/6 | | 30/6 | | | | |
|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | 5AM | Roje | 5AM | Roje | 5AM | Roje | STOP | | |
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | | | | | | | |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign |

## Regular Medication

IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 29/6 | SYP CALCIMAX PLUS | 2.5 ml | oral | 12th hrly | | |

| Date | 29/6/26 | | 30/6 | | 1/7 | | | | |
|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | | | 6AM | Roje | 6AM | Roje | | | |
| TIME GIVEN | 6pm | Roje | | | | | | | |
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | 6pm | | 6PM | | | | |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign |

## Regular Medication

IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 30/06 | T. BENADON 40mg | 2 Tab | oral | one daily | (80 mg) | |

| Date | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | Stop | | | | | | |
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | | | | | | | |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign |

## Regular Medication

IP No:

| Start Date / Time | Drug | Dose | Route | Frequency | Instructions if any | Prescribed by Dr. Sign |
|---|---|---|---|---|---|---|
| 01/07 | SYP GARDENAL | 1.5 ml | oral | 12th hourly | 20mg/5ml | |

| Date | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | | | | | | | |
| TIME GIVEN | | | | | | | | | |
| Authenticated Dr. : | Sign | | Sign | | Sign | | Sign | | Sign |

# NURSING CARE PLAN

Patient Name: B/o vagadani Age/Sex: male DOA: 27/6/26 MR No.: 162608635

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 28/6/26 | Assess the General condition | -> To provide warm Care. | Self Given | -> provided warm Care. | Belg iron on Support with 0.5 litre | Roja 10636 |
| | | -> To provide comfortable position | | -> provided comfortable position | | |
| | | -> Apply dry fluid. | | -> vital checked recorded HR:- 132/mt SpO2 95%. | | |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 · AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# NURSING CARE PLAN

ankura hospital for women & children

9M

Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Pat Age/Sex: DOA: MR No.:

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 28/6/26 6am-2pm | on NPO<br>Seizures | -To provide warm care<br>To provide comfortable position<br>To monitor vitals | Risk for unstable blood glucose level<br>risk for injury related to seizure activity<br>risk for impaired cerebral tissue perfusion | -Vitals checked and recorded<br>-provided warm care<br>-provided comfortable position | -NPO<br>-No seizures<br>-Tolerance feeds well | Anasuya 12222 |

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

# NURSING CARE PLAN

Patient Name: _____ Age/Sex: _____ DOA: _____ MR No.: _____

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 28/06/26 2pm to 8pm | - Baby is on IVF 10%D+5D<br>- TF 15ml and 1hly | - Asses the general condition<br>- To monitor Vitals<br>- Provide Comfortabe Position<br>- Monitor Seizure activity<br>- Administer medication as per doctors order | Deficient fluid volume R/t inability to tolerating feed. | - Assesed the general condition<br>- vitals checked recorded<br>- provided comfortable position<br>- monitored Seizure activity<br>- Administered medication as per doctors Order | Baby still improving | Shanty 2684. |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 · AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# NURSING CARE PLAN

Patient Name: B/o vagadani Age/Sex: male DOA: 24/6/26 MR No.: 162608635

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 24/6/26 6pm 6am | Assess the General condition | -> To provide warm Cae. | Seizure | -> provided warm Cae. | Brg ison on Support with 0.5 ltrs | Roja 10636 |
| | | -> To provide comfortable position | | -> provided comfortable position | | |
| | | -> Appo. IV fluid. | | -> vital checked Recorded HR:- 132/mt SpO2 95%. | | |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ Age/Sex: _____ DOA: _____ MR No.: _____

# NURSING CARE PLAN

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 29/06/26 8Am to 2pm | - Lethargy<br>- on IV Fluids<br>- I/F 15 ml and hrly | - Asses the general condition<br>- To monitor vitals<br>- To provide and hrly feed<br>- Administer medication as per doctors Order<br>- Baby is on IVF 10%D | Impaired breathing pattern R/t disease condition | - Assesed the general condition<br>- monitor vitals & recorded<br>- provided and hrly feed<br>- medication given as per doctors order<br>- Baby continue on IVF. | Baby still improving | Shanmu 0834. |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 · AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patien _____ Age/Sex: _____ DOA: _____ MR No.: _____

# NURSING CARE PLAN

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 29/6/26 2pm to 8p | => on my ob -scvation baby is on Room air Rv. Fluids Stop. | => Continued Coarm care => Spoon feed Continue => NGt done => Neurologist opinion done => MRI Im Bill paid => GRBS 6th | => Risk of Injury due to Seizures | => 70 provided Comfortable possition => spoon feed 60ml accepted => Abx given as per doctor advice GRBS 6thly | => Still on Room air Rv fluid Support | Sth 2UG |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ DOA: _____ MR No.: _____

# NURSING CARE PLAN

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 27/6/26 8pm to 8am | Assess the General condition | -> To provide warm care. | -> Risk of Injury due to seizures | -> provide warm care. | Baby is on Room air | Raje 10636 |
| | | -> To provide comfortable position | | -> provide comfortable position | | |
| | | -> s/f advise. | | -> vital checked & recorded HR: 142/min Spo2: Temp 36.5°. | | |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# NURSING CARE PLAN

Patient Name: Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Age/Sex: _____ DOA: _____ MR No.: _____

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 28/6/26 8am noon pm | => on my obser -vation baby is on room air | => Continue Care => Spoon fed supted => Abx continu => Add to plan today => NO New Complaint | => Dual activity due to disease condition as evidenced | => To provided Comfortable position => Spoon fed supted | => Stable on Room air | Smitha RNC, |

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

# NURSING CARE PLAN

Patient Name: _____ Age/Sex: _____ DOA: _____ MR No.: _____

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 30/06/26<br>2pm to 8pm | - Lethargy<br>- on IV Fluids<br>- 30 ml 2nd hrly | - Asses the general condition<br>- To monitor vitals<br>- To provide 2nd hrly feed<br>- Administer medication as per doctors Order<br>- Baby is on IVF 10%D | Impaired breathing pattern R/t disease condition | - Assesed the general condition<br>- monitor vitals & recorded<br>- provided 2nd hrly feed<br>- medication given as per doctors order<br>- Baby continue on IVF. | Baby still improving | Shanmu 0634. |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 · AHWC/IP/F/05

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

# NURSING CARE PLAN

Patient Name: _____ DOA: _____ MR No.: _____

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 30/6/26 8pm to 8am | Assess the General condition | -> To provide warm care. | -> Risk of Injury due to seizures | -> provide warm care. | Baby is on Room air | Raje 10636 |
| | | -> To provide comfortable position | | -> provide comfortable position | | |
| | | -> s/f adms. | | -> vital checked & recorded HR:- 142/min Sp-95% Temp 36.5°C. | | |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 AHWC/IP/F/05

ankura hospital for women & children

9M

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# NURSING CARE PLAN

Patient Name: _____ Age/Sex: _____ DOA: _____ MR No.: _____

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| | | | | | | |

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient N _____ Age/Sex: _____ DOA: _____ MR No.: _____

# NURSING CARE PLAN

| Date & Time | Nursing Assessment | Planning | Diagnosis | Implementation | Evaluation | Nurse Name & Sign |
|---|---|---|---|---|---|---|
| 4of/26 Rem to 2pm | => on my obser -vation baby is on room air | => Continew Co arm care => Spoon feed cuepted => Abx continu => Gardinal Syp. today Start => NO New Complaint | => Dual activity due to discas condition as evidenced | => To provided Comfortable possition => Spoon feed acpted => Continuw Abx given asper doctor advice => plan to Shift to mother said | => Still on Room aur | Sithq EUG. |

ANKURA HOSPITAL FOR WOMEN AND CHILDREN · REVISION NO.: 3 | ISSUE NO.: 4 AHWC/IP/F/05

ankura hospital for women & children

9M

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# NURSES NOTES

ankura hospital for women & children | 9M

Patient Name: B/O Vagadavi MR No: 162608635

DOB/Age: 4dy Sex: male Ward/Room No.: NICU

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 27/6/24 | Night duty Report | |
| 9pm | -> Hand over taken from c/k staff | Roja 10636 |
| | -> Assess the General condition | |
| 10pm | -> Babyson O2 support @ 0.5 lpm. | |
| | -> Babyson NJPO FIU fluid | Roja 10636 |
| 11pm | -> IV fluid 10% + 5D @ 13.7ml | |
| | -> urine passed | |
| 12Am | -> stool not passed | |
| | -> CBP CRP Blood Group. VBG | Roja 10636 |
| | -> Blood cls T. Calcium, RBS | |
| 1Am | CUE, RFT, LFT, Send to lab @ done. | |

# NURSES NOTES

Patient Name: B/O vagdevi MR No: 162608635

DOB/Age: 4 day Sex: Male Ward/Room No.: NICU

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| | -> NS 20 ml bolus given. | |
| 8 AM | -> Inj. Vit K. 1mg given. | Rose 10636 |
| | -> Inj. Torsemide given. | |
| | -> Inj. Levipil loading given. | |
| 4 AM | -> Inj. phenobarbital loading given. | Rose 10636 |
| | -> inj phenobarbital loading given. | |
| | -> All bs clubbed recorded | |
| 5 AM | -> vital clubbed recorded | Rose 10636 |
| | HR: 124/min SpO2 95% Temp 36.5°C | |
| | -> plan. G & G. abs (+) dec. | |
| 6 AM | -> a/o others given. | Rose 10636 |
| 8 AM | -> Hand over given. morning duty Staff | |

# NURSES NOTES

ankura hospital for women & children | 9M

Patient Name: _____ MR No: _____

DOB/Age: _____ Sex: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 28/6/26 | mrng Duty Report | |
| 8am | - case hand over taken from night duty sister Rojo by Anasuya | Anasuya 12277 |
| 8.15am | - Assess the General condition of the baby | |
| 8.30am | - Baby is under radiant warmer<br>- baby was activity good<br>- on NPO. 0.5 litres/per/min | Anasuya 12277 |
| 9am | - Baby is on NPO till further orders<br>- vitals checked and recorded<br>- baby was not passed<br>- urine monitoring | Anasuya 12277 |
| 10am | - UVC inserted. 5no. fix at 9cm<br>- Amonpa, urine for reducing substency | Anasuya 12277 |
| 11am | - x-Ray done for UVC<br>- Treatment given as per doctor order | Anasuya 12277 |

# NURSES NOTES

Patient Name: _____ MR No: _____

DOB/Age: _____ Sex: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| | —TV. 150ml/kg/day. 5% Dextrose + 10% Dextrose<br>—Inj: Calcium 4.5ml + 4.5ml 6th hrly<br>— an antibiotic, Toxim<br>— Inj: Gardinal, Leviphil,<br>— EEG done, NSG due<br>— plan for Neurologic opinion<br>— start. TF. 10ml 2nd hourly mom<br>— Baby was activity good<br>— NO further seizures<br>— GRBS monitor 6th hourly<br>— No other any issues<br>— hand over given to Evng duty staff | Anasuya 12272<br>Anasuya 12272<br>Anasuya 1227 |

# NURSES NOTES

ankura hospital for women & children | 9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Sex: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| | Evening Duty Report on 28/06/26 | |
| 2:00 pm | — Hand over taken from SIN Anasuya by Shancy | Shancy 2684 |
| | — Baby under warmer connected c cardiac monitor | |
| 8:00 pm | — vitals checked & recorded HR: 128 bmt SpO2: 96% Temp: 36.5°C | Shancy 2684 |
| 4:00 pm | — Baby is on low flow O2 | |
| | — IVF 10% D + 5D → 13.7 ml/hr on flow | Shancy 2684 |
| 5:00 pm | — Doctors rounds done | |
| | — I/O chart maintained | |
| | — TPR chart maintained | |
| 6:00 pm | — All due medication given as per doctors order<br>inj. optineuron OD<br>inj. devipil 25 mg BD<br>inj. phenobarbitone 5-5 mg BD<br>inj. cal-gluconate 6 6 H<br>inj. Taxim BD continue | Shancy 2684 |
| | — Baby active & good | |
| | — TF 10 ml 2nd hrly EBM given. | Shancy 2684 |
| 8:00 pm | — Hand over given to night duty SIS: by Shancy | |

# NURSES NOTES

ankura hospital for women & children

9M

Patient Name: Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR No: _____

DOB/Age: _____

Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 28/6/26 | Night duty Report | |
| 8pm | -> Hand over taken from Evening duty staff | |
| 10pm | -> Assess the General condition<br>-> Baby is on with O2 Support on ventilator.<br>-> Baby is on T/F 10 f 2nd lg. | |
| 11pm | -> Urine passed<br>-> All medication given as per doctor orders. | |
| 12am | -> Inj. optineuron OD<br>-> Inj. Levipil BD<br>-> Inj. phenobarbitone - BD<br>-> Inj. Tonefos BD<br>-> Inj. Cal. gluconate 6th<br>-> | |

# NURSES NOTES

ankura hospital for women & children | 9M

Patient Name: _____ MR No: _____

DOB/Age: _____ Ward/Room No.: _____

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

| Date / Time | Notes | Name of the nurse / ID No. & Sign |
|---|---|---|
| 1AM | -> Warm Cone.<br>-> vital checked & recorded<br>HR: 142/min SpO2 95% Temp 36.8°C | Roja 10636 |
| 2AM | -> CRT 6/3 sec.<br>-> Baby is on RA & lying in cradle.<br>-> Diaper changed. | Roja 10636 |
| 4AM | -> no other issues.<br>-> Bathing given.<br>-> vitals checked & recorded | Roja 10636 |
| 5AM | -> no fresh seizures<br>-> NSG. Tx done. | Roja 10636 |
| 8AM | -> Hand over given. morning duty Staff | Roja 10636 |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Sex: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 8:00 AM | Morning Duty Report on 29/06/26<br>— Hand over taken from SIN Roja by Shancy | Shancy 2634 |
| 9:00 AM | — Baby under warmer connected c cardiac monitor<br>— Vitals checked & recorded<br>HR: 128 bmt SpO2: 96 %. Temp: 36.5°C | Shancy 2634 |
| 10:00 AM | — Baby is on room air<br>— Baby taking 15 ml slf and bmlk.<br>Baby feed accepting well<br>— IUF 10% D- 10.5 ml 1hr on dflow | Shancy 2634 |
| 11:00 AM | — Doctors rounds done by Ds. S. murki<br>— All due medication given as per doctors Order<br>inj. Optineuron<br>inj. Taxim<br>Oral calcimax plus<br>inj. pheno barbitone | Shancy 2634 |
| 12:00 PM | — Baby passed urine & stool<br>— No Fresh issues | Shancy 2634 |
| 2:00 PM | — Hand over given to evening duty SIS: by Shancy | Shancy 2634 |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 29/6/26 2pm to 8pm | Evening duty Report<br>⇒ Case hand over taken from Sr Shavy by Sunitha | Sunitha 2U61 |
| | ⇒ Assessed the genaral condition of the baby kept on under warmer & monitor | |
| | ⇒ Baby is on room air | |
| 3pm | ⇒ Baby vitals checked & Recorded | Sunitha |
| | ⇒ Temp - 36.5 °F · HR - 146 · SpO2 96 RR - 46 | |
| | ⇒ maintand the BP chart & Recorded | |
| | ⇒ Baby passed urine and stool | |
| 4pm | ⇒ NSU done by Suresh Sir. | Sunitha 2U61 |
| | ⇒ Neurologist opinion done dy dr Keerthi Kandhana madam | |
| | ⇒ MRI & contrast Bill raised bill paid Test dun Tmr plan | |
| 6pm | ⇒ Continue. Syp. Calcimax P - 4j · Phenobarbitone. | Sunitha 2U61 |
| | ⇒ Evening Rounds done | |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Na _____ MR No: _____

DOB/Age: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 1pm | => BP monitoring<br>=> RBS monitoring<br>=> no new complaints<br>=> | |
| 8pm | => case hand over given to night duty Sr by Smitha | 2116 |

# NURSES NOTES

ankura hospital for women & children | 9M

Patient Name: Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR No: _____

DOB/Age: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 25/6/26 | night duty Report | |
| 8pm | -> Hand over taken from twing dysatt | Rose 10636 |
| | -> Assess the Genral condition | |
| 10pm | -> Baby is on Room air | |
| | -> vital clubed recorded | Rose 10636 |
| 11pm | HR:- 123/mt Sp2 95%. Tap 36.8° | |
| | -> Baby is on s/cradles. | |
| | -> uniform parced | |
| 12am | -> All medication given as per doctor order. | Rose 10636 |
| | -> Tab: Tarcim BD | |
| | Tab:- Gandinal BD | |
| 1Am | Tab:- optineuron OD | Rose 10636 |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 3 AM | -> oral calcimasp<br>-> strep developed<br>-> Neurologist opinion done.<br>-> plan. MRI Brain | Roja 10636 |
| 4 AM | -> GRBS 6/5 Cy.<br>-> warm care. | Roja 10636 |
| 5 AM | -> IV cannula preset Good<br>-> IV fluid 10% @ 10.5 ml Cy. | Roja 10636 |
| 6 AM | -> Bathing given.<br>-> wt. checked recorded | Roja 10636 |
| 8 AM | -> hand over given. morning duty staff | Roja 10636 |

# NURSES NOTES

ankura hospital for women & children | 9M

Patient Name: Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR No: _____

DOB/Age: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 30/6/26 8am to 2pm | Morning duty Report-<br>=> Case hand over taken from Sr Roja by Sunitha<br>=> Assessed the general condition of the baby kept on under warmer & monitor | Sunitha 246 |
| 10AM | => Baby is on room air<br>=> Baby vitals checked & Recorded<br>=> Temp-36.5 HR-146 Spo2 96 RR-46<br>=> maintained the BP chart & Recorded<br>=> Baby passed urine and stool | Suma |
| 12pm | => Dr-Srinivas Sir Rounds done<br>=> Today Tab- Pyradoxine - 80 mg to be give<br>=> MRI & contrast Bill raised bill paid Test done | Sunitha 246 |
| 1pm | => Continue- Syp. Calcimax P - 4j - Phenobarbitone. | Sunitha 246 |
| 2pm | => Case handover given to evening duty Sr. Shaney by Sunitha | |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Sex: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 8:00 AM | Morning Duty Report on 30/06/26<br>— Hand over taken from SIN sunitha by Shancy | Shancy 2634 |
| 9:00 AM | — Baby under warmer connected c cardiac monitor<br>— vitals checked & recorded<br>HR: 128 bmt SpO2: 96 %. Temp: 36.5°C | Shancy 2634 |
| 10:00 AM | — Baby is on room air<br>— Baby taking 30 ml slf and bmlk.<br>Baby feed accepting well<br>— IVF 10% D- 3.5 ml /hr on dflow | Shancy 2634 |
| 11:00 AM | — Doctors rounds done<br>— All due medication given as per doctors Order<br>Tab. pyridoxine 80 mg OD, stat given & cont<br>inj. Taxim<br>Oral calcimax plus<br>inj. pheno barbitone | Shancy 2634 |
| 12:00 PM | — Baby passed urine & stool<br>— MRI done. | Shancy 2634 |
| 2:00 PM | — Hand over given to evening duty sis: by Shancy | Shancy 2634 |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Sex: _____ Ward/Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| | Morning Duty Report on 30/06/26 | Shancy 2634 |
| 2:00 pm | — Hand over taken from SIN sunitha by Shancy | |
| | — Baby under warmer connected c cardiac monitor | Shancy 2634 |
| 3:00 pm | — vitals checked & recorded HR: 128 bmt SpO2: 96 %. Temp: 36.5°C | |
| | — Baby is on room air | Shancy 2634 |
| 4:00 pm | — Baby taking 30 ml slf and bmlk. Baby feed accepting well | |
| | — IUF 10% D- 3.5 ml /hr on dflow | |
| | — Doctors rounds done | Shancy 2634 |
| 6:00 pm | — All due medication given as per doctors Order Tab. pyridoxine 80 mg OD, stat given & cont inj. Taxim Oral calcimax plus inj. pheno barbitone | |
| 7:00 pm | — Baby passed urine & stool | Shancy 2634 |
| | — MRI done. | |
| 8pm | — Hand over given to evening duty sis: by Shancy | Shancy 2634 |

# NURSES NOTES

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: _____ MR No: _____

DOB/Age: _____ Room No.: _____

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 30/6 | Night duty Report | |
| 8pm | -> Hand over taken from Evening duty | Roje 10636 |
| 10pm | -> Assess the General condition<br>-> Baby is on room air<br>-> vital stable passed | |
| 11pm | -> Baby is on c/feeds.<br>-> All medication given. | Roje 10636 |
| 12am | as per doctor order. | |
| 2am | => Sup Evening<br>-> Inj. phenobarbitone continue | Roje 10636 |
| 1am | -><br>-> GCNS monitoring 6 hrly<br>-> nil per os. | |

# NURSES NOTES

ankura hospital for women & children

9M

Patient Name: _____ MR No: _____

DOB/Age: _____ Room No.: _____

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 2AM | -> Vital checked recorded<br>HR:- 142/min SpO2 98% Temp 36.2<br>-> warm Care. | Rafe 10636 |
| 3AM | -> also other issue.<br>-> Bathing given. | Rafe 10636 |
| 7AM | -> wt checked recorded<br>-> CRPs checked recorded | |
| 8AM | -> Hand over given. morning day Staff | Rafe 10636 |

# NURSES NOTES

ankura hospital for women & children

9M

Patient Name: Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR No:

DOB/Age:

Ward/Room No.:

| Date / Time | Nurses Note | Name of the nurse / ID No. & Sign |
|---|---|---|
| 27/06/26 8am to 2pm | Morning * duty * Report<br>=> Case hand over taken from Sr Roja by Sunitha | |
| | =>Assessed the genaral condition of the baby baby kept on under warmare monitor | |
| 10am | => Baby is on Room air<br>=> Baby vitals checked & Recorded<br>Temp-36.5 HR-146 . Spo2-96 RR-46. | |
| 12pm | =>Dr. Karthick Sir Rounds done<br>=> Spoon fed 30ml and bby accepted | |
| | =>Continued. and start Cardinal Syrup. | |
| | =>no desaturation. | |
| 4pm | => no new Complaints | |
| | =>Case hand over given to Evening duty Sr. by Sunita | |

# INTAKE & OUTPUT CHART

ankura hospital for women & children

9M by ankura hospital Birth & beyond

28/6/26

Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patien | Gender / Age: | MR. No.:

DOA: | Term / Preterm: | CGA: wks days

Past 2 g/day | Past 24 hrs.Output:

Total Volume: ml/kg/day
ml/hr
OG/NG/ORA: ml
hrly
I.V.: ml/hr
Other:

Urine:
OG/NG Aspiration:
Stool:
Other:

Admit Wt.:
Yest. Wt.:
Total Wt.:
H.C:

| | INTAKE | | | | | OUTPUT | | |
|---|---|---|---|---|---|---|---|---|
| | IV | IV/ORAL | | ORAL | | | | |
| | Type of Fluid | Hrly Total | Others (medication /Blood) | Feeds | Total | Urine | Stool | N.G. Asp/ VOMIT |
| | IVF. 10% Dextrose + 5% Dextrose | | | | | | | |
| 8 am | 13.7 ml | | | NPO | | 5Am | | |
| 9 am | 13.7 ml | | | NPO | | to | | |
| 10 am | 13.7 ml | | | NPO | | 2pm | | |
| 11 am | 13.7 ml | | | NPO | | | | |
| 12 am | 13.7 ml | | | | | 120 ml | | |
| 1 pm | 13.7 ml | | | | | | | |
| 2 pm | 13.7 ml | | | 10 ml | | | | |
| 3 pm | 10.0 | | | | | | | |
| 4 pm | 10.0 | | | 15 ml | | | | 15ml |
| 5 pm | 10.0 | | | | | | | |
| 6 pm | 10.0 | | | 10ml | | | | |
| 7 pm | 10.0 | | | | | 70ml | | |
| 8 pm | 10.0 | | | 10ml | | | | |
| 9 pm | 10.0 | | | | | | | |
| 10 pm | 10 0f | | | T/F 10f | | | | |
| 11 pm | 10 0f | | | | | | | |
| 12 am | 10.0 f | | | T/F 10f | | | | |
| 1 am | 10 0f | | | | | | | |
| 2 am | 10.0f | | | T/F 10f | | | | |
| 3 am | 10.0f | | | | | | | |
| 4 am | 10.0f | | | T/F 10f | | | | |
| 5 am | 10.0f | | | | | | | |
| 6 am | 10.0f | | | T/F 10f | | | | |
| 7 am | 10.0f | | | | | | | |
| TOTAL | | | | | | | | |

# INTAKE & OUTPUT CHART

ankura hospital for women & children

9M by ankura hospital Birth & beyond

24/6/26

Patient Name: B/o Vagadevi Gender / Age: 4 day MR. No.: 16260863

DOA: 20/6/26 Term / Preterm: CGA: wks days

Past 24 hrs. Intake: ml/kg/day Past 24 hrs.Output:

| | | | |
|---|---|---|---|
| Total Volume: | ml/kg/day | Urine: | Admit Wt.: |
| ml/hr | | OG/NG Aspiration: | Yest. Wt.: |
| OG/NG/ORA: | ml | Stool: | Total Wt.: |
| hrly | | Other: | H.C: |
| I.V.: | ml/hr | | |
| Other: | | | |

| | INTAKE | | | | | OUTPUT | | |
|---|---|---|---|---|---|---|---|---|
| | IV | IV/ORAL | | ORAL | | | | |
| | Type of Fluid | Hrly Total | Others (medication /Blood) | Feeds | Total | Urine | Stool | N.G. Asp/ VOMIT |
| | | | | | | | | |
| 8 am | | | | 1 | | | | |
| 9 am | | | | | | | | |
| 10 am | | | | N | | | | |
| 11 am | | | | | | 9pm | | |
| 12 am | | | | P | | | | |
| 1 pm | | | | | | 3ml | | |
| 2 pm | | | | | | 5ml | | |
| 3 pm | | | | O | | | | |
| 4 pm | | | | | | | | |
| 5 pm | | | | 1 | | | | |
| 6 pm | 10%+5D | | | | | | | |
| 7 pm | | | | | | | | |
| 8 pm | 13.7 fp | | | | | | | |
| 9 pm | 13.7 fp | | | | | | | |
| 10 pm | 13.7 fp | | | | | | | |
| 11 pm | 13.7 fp | | | | | | | |
| 12 am | 13.7 fp | | | | | | | |
| 1 am | 13.7 fp | | | | | | | |
| 2 am | 13.7 fp | | | | | | | |
| 3 am | 13.7 fp | | | | | | | |
| 4 am | 13.7 fp | | | | | | | |
| 5 am | 13.7 fp | | | | | | | |
| 6 am | 13.7 fp | | | | | | | |
| 7 am | 13.7 fp | | | | | | | |
| TOTAL | | | | | | | | |

stool not passed

# INTAKE & OUTPUT CHART

ankura hospital for women & children

9M by ankura hospital Birth & beyond

29/06/26

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

| | | | |
|---|---|---|---|
| Patient Name: | | Gender / Age: | MR. No.: |
| DOA: | | Term / Preterm: | CGA: wks days |
| Past 24 hrs. Intak | | Past 24 hrs.Output: | |

Total Volume: ml/kg/day
ml/hr
OG/NG/ORA: ml
hrly
I.V.: ml/hr
Other:

Urine:
OG/NG Aspiration:
Stool:
Other:

Admit Wt.:
Yest. Wt.:
Total Wt.:
H.C:

| | INTAKE | | | | | OUTPUT | | |
|---|---|---|---|---|---|---|---|---|
| | IV | IV/ORAL | | ORAL | | | | |
| | Type of Fluid | Hrly Total | Others (medication /Blood) | Feeds | Total | Urine | Stool | N.G. Asp/ VOMIT |
| | 10% D | | | | | | | |
| 8 am | 10.5 | | | | | | | |
| 9 am | 10.5 | | | S/F- EBM | 15ml | ✓ | | |
| 10 am | 10.5 | | | | | | | |
| 11 am | 10.5 | | | S/F - mom | 15 ml | | | |
| 12 am | 10.5 | | | | | | | |
| 1 pm | 10.5 | | | S/F - mom | 15ml | ✓ | ✓ | |
| 2 pm | 10.5 | | | | | | | |
| 3 pm | 10.5 | | | S/F mom | 15ml | ✓ | ✓ | |
| 4 pm | 10.5 | | | | | | | |
| 5 pm | 10.5 | | | S/F mm | 15ml | ✓ | | |
| 6 pm | 10.5 | | | | | | | |
| 7 pm | 10.5ml | | | S/F mom | 20ml | ✓ | | |
| 8 pm | 10-5 ml | | | | | | | |
| 9 pm | 10.5 | | mom | S/F | 20ml | | | |
| 10 pm | 10.5 | | | | | | | |
| 11 pm | 10.5 | | | S/F | 25ml | - | ✓ | |
| 12 am | 10.5 | | | | | | | |
| 1 am | 10.5 | | mom | S/F | 25ml | | | |
| 2 am | 10.5 | | | | | | | |
| 3 am | 10.5 | | | S/F | 25ml | ✓ | | |
| 4 am | 10.5 | | | | | | | |
| 5 am | 10.5 | | mom | S/F | 25ml | | | |
| 6 am | 10.5 | | | | | | - | |
| 7 am | 10.5 | | | S/F | 25ml | ✓ | | |
| TOTAL | | | mom | | | | | |

# INTAKE & OUTPUT CHART

ankura hospital for women & children

9M by ankura hospital Birth & beyond

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: Gender / Age: MR. No.:

DOA: Term / Preterm: CGA: wks days

Past 24 hrs. Past 24 hrs.Output:

Total Volume: ml/kg/day
ml/hr
OG/NG/ORA: ml
hrly
I.V.: ml/hr
Other:

Urine: Admit Wt.:
OG/NG Aspiration: Yest. Wt.:
Stool: Total Wt.:
Other: H.C:

30/6/26

| | INTAKE | | | | | OUTPUT | | |
|---|---|---|---|---|---|---|---|---|
| | IV | IV/ORAL | | ORAL | | | | |
| | Type of Fluid | Hrly Total | Others (medication /Blood) | Feeds | Total | Urine | Stool | N.G. Asp/ VOMIT |
| | | | | | | | | |
| 8 am | | | | | | | | |
| 9 am | | | | S/F | 20ml | ✓ | ✓ | |
| 10 am | | | | | | | | |
| 11 am | | | | S/F | 30ml | ✓ | ✓ | |
| 12 am | | | | | | | | |
| 1 pm | | | | S/F | 30ml | ✓ | | |
| 2 pm | | | | | | | | |
| 3 pm | | | | S/F | | | | |
| 4 pm | | | | | | | | |
| 5 pm | | | | S/F | | | | |
| 6 pm | | | | | | | | |
| 7 pm | | | | | | | | |
| 8 pm | | | | S/F | 30ml | | — | |
| 9 pm | | | | | | | | |
| 10 pm | | | | S/F | 30ml | ✓ | | |
| 11 pm | | | | | | | | |
| 12 am | | | | S/F | 30ml | | | |
| 1 am | | | | | | | ✓ | |
| 2 am | | | | S/F | 30ml | ✓ | | |
| 3 am | | | | | | | | |
| 4 am | | | | S/F | 30ml | | | |
| 5 am | | | | | | | | |
| 6 am | | | | S/F | 30ml | ✓ | | |
| 7 am | | | | | | | | |
| TOTAL | | | | | | | | |

# INTAKE & OUTPUT CHART

ankura hospital for women & children

9M by ankura hospital Birth & beyond

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Name: Gender / Age: MR. No.:

DOA: Term / Preterm: CGA: wks days

Past 24 hrs. Intake: ml/kg/day Past 24 hrs.Output: 01/7/2026

Total Volume: ml/kg/day
ml/hr
OG/NG/ORA: ml
hrly
I.V.: ml/hr
Other:

Urine:
OG/NG Aspiration:
Stool:
Other:

Admit Wt.:
Yest. Wt.:
Total Wt.:
H.C:

| | INTAKE | | | | | OUTPUT | | |
|---|---|---|---|---|---|---|---|---|
| | IV | IV/ORAL | | ORAL | | | | |
| | Type of Fluid | Hrly Total | Others (medication /Blood) | Feeds | Total | Urine | Stool | N.G. Asp/ VOMIT |
| | | | | | | | | |
| 8 am | | | | | | | | |
| 9 am | | | Formula | Spoon ft | 30 ml | ✓ | ✓ | |
| 10 am | | | | | | | | |
| 11 am | | | Formula | S/F | 30 ml | | | |
| 12 am | | | | | | | | |
| 1 pm | | | Formula | S/F | 30 ml | | | |
| 2 pm | | | | | | | | |
| 3 pm | | | | | | | | |
| 4 pm | | | | | | | | |
| 5 pm | | | | | | | | |
| 6 pm | | | | | | | | |
| 7 pm | | | | | | | | |
| 8 pm | | | | | | | | |
| 9 pm | | | | | | | | |
| 10 pm | | | | | | | | |
| 11 pm | | | | | | | | |
| 12 am | | | | | | | | |
| 1 am | | | | | | | | |
| 2 am | | | | | | | | |
| 3 am | | | | | | | | |
| 4 am | | | | | | | | |
| 5 am | | | | | | | | |
| 6 am | | | | | | | | |
| 7 am | | | | | | | | |
| TOTAL | | | | | | | | |

# LAB INVESTIGATION CHART

ankura hospital for women & children

9M by ankura hospital

Patient Name: B/O vagadevi MR. No.: 608635

DOB/Age: 04 Gender: Male Ward/Room No.: NICU Blood Group:

Rh Type: HIV: HBsAg: HCV:

| Date | 27/6 | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|
| 1. Hematology | 15.6 | | | | | | | | |
| * HB% | 1.5.6 | | | | | | | | |
| * PCV/HCT | 49.8 | | | | | | | | |
| * WBC | 9100 | | | | | | | | |
| * DC-NP | | | | | | | | | |
| * DC-LC | | | | | | | | | |
| * Platelet count | 68000 | | | | | | | | |
| * Peripheral Smear | | | | | | | | | |
| * Reticulocyte count | | | | | | | | | |
| * Band Count | | | | | | | | | |
| 2. CRP | | | | | | | | | |
| 3. Electrolytes | 2.4. | | | | | | | | |
| * Na+ | 140 | | | | | | | | |
| *K+ | 5.1 | | | | | | | | |
| *Cl+ | 107 | | | | | | | | |
| *Ca2+ | 5.0.6 | | | | | | | | |
| *Po4 | | | | | | | | | |
| *Mg2+ | | | | | | | | | |
| *HCO3 | | | | | | | | | |
| *Anion gap | | | | | | | | | |
| 4. Renal Function Tests | | | | | | | | | |
| *Urea | 20 | | | | | | | | |
| *BUN | | | | | | | | | |
| *Creatinine | 0.4 | | | | | | | | |
| *Calculated GFR | | | | | | | | | |
| FENa | | | | | | | | | |
| 5. Liver Function Tests | 15.5 | | | | | | | | |
| *Total Bilirubin | | | | | | | | | |
| *Direct Bilirubin | 1.6H | | | | | | | | |
| *Indirect Bilirubin | 13.0 | | | | | | | | |
| *AST | 35 | | | | | | | | |
| *ALT | 214 | | | | | | | | |
| *ALP | | | | | | | | | |
| *GGT | | | | | | | | | |
| *Total Protein | 5.2 | | | | | | | | |
| *Albumin | 3.4 | | | | | | | | |
| *PT/INR | | | | | | | | | |

# VITALS MONITORING CHART

ankura hospital for women & children

9M

MR No. :
Patient Na
Age :
Sex :

Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Current Weight:
Current Height:
Blood Group & Rh Type:
Date : 27/6/26

Time
Month
Day
Hours
Minutes

Temperature
0C
42.0
Of 41.5
105.8 41.0
40.5
104.0 40.0
39.5
102.0 39.0
38.5
100.4 38.0
37.5
98.6 37.0
36.5

Pulse Per Minute
96.8 36.0
35.5
95.0 35.0
170 34.5
160 34.0
150 33.5
140 33.0

Respiration Per Minute
130
120
110
100
90
80
70
60
50
40
30
20
10

9 10 11 12 1 2 3 4 5 6 7 8
PM PM PM AM AM AM AM AM AM AM AM AM

365 365 365 365 368 365 366 36

140 132/88 125/78 125 110 132 140

92 98 92 96 98 92 95 92

Bllod Pressure mmHg
250
240
230
220
210
200
190
180
170
160
150
140
130
120
110
100
90
80
70
60
50
40
30
20
10

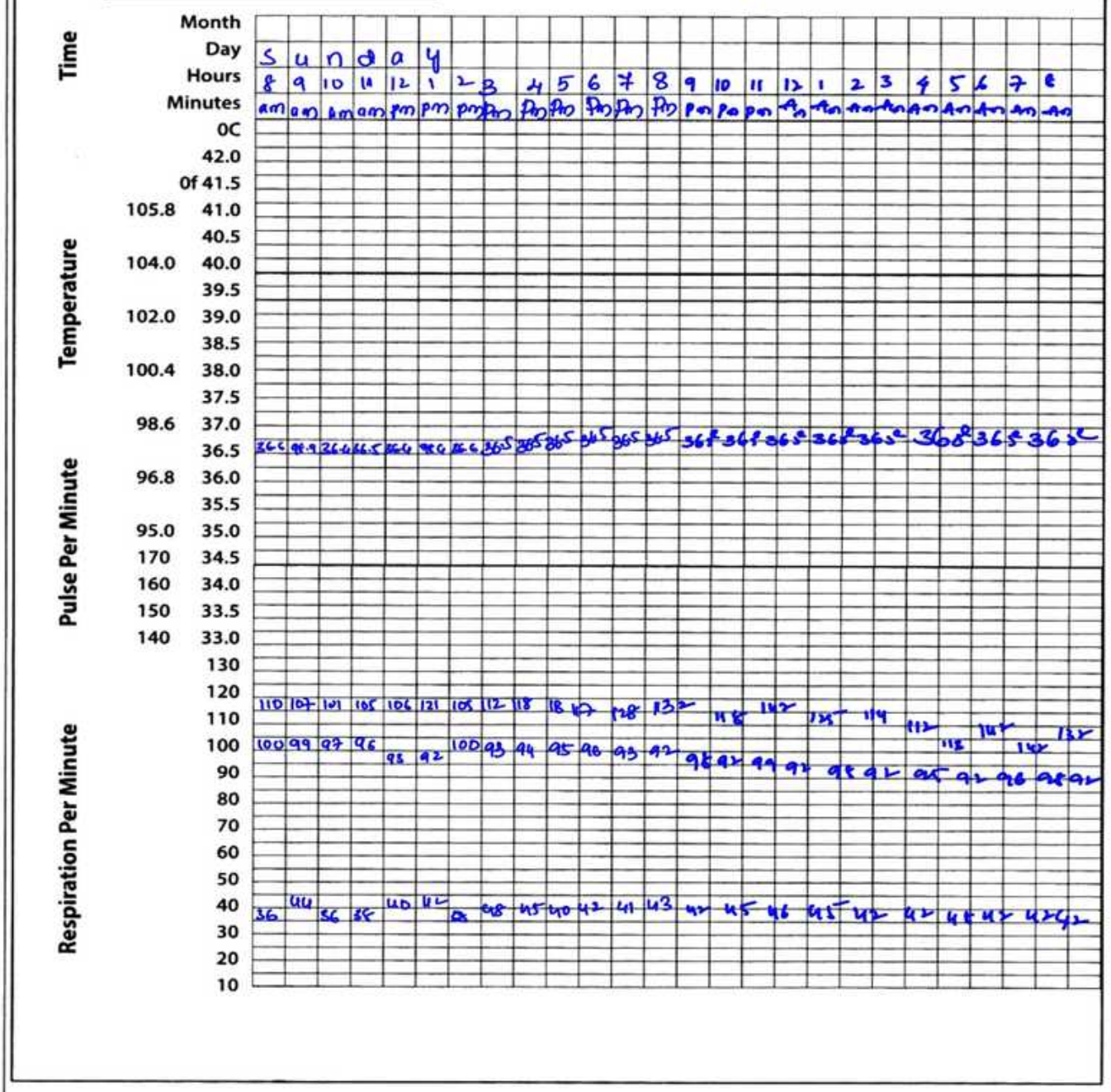
# VITALS MONITORING CHART

ankura hospital for women & children

9M

MR No. :
Patient Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : Age : 4 D Gender : Male
Sex : Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Current Weight:
Current Height:
Blood Group & Rh Type:
Date : 28/6/26



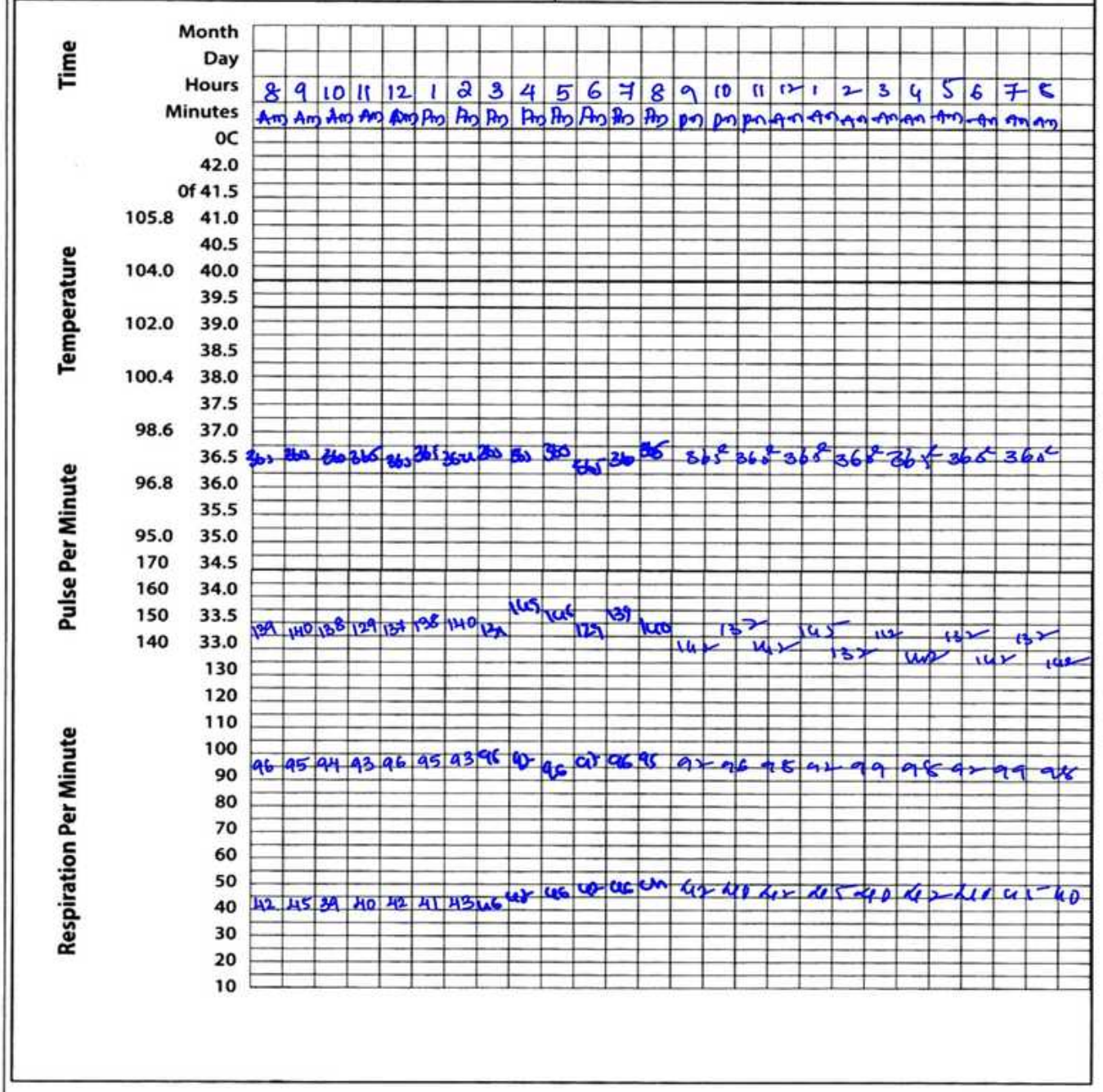
# VITALS MONITORING CHART

ankura hospital for women & children

9M

| | |
|---|---|
| MR No. : | Name : VAGDEVI<br>UHID NO : 162608635 |
| Patient Name : | IPD NO : LB26/3007 |
| Age : | Age : 4 D Gender : Male |
| Sex : | Consult : DR. SRINIVAS MURKI<br>Adm Date : 27-06-2026 09:03 PM |

Current Weight:
Current Height:
Blood Group & Rh Type:
Date : 29/06/26



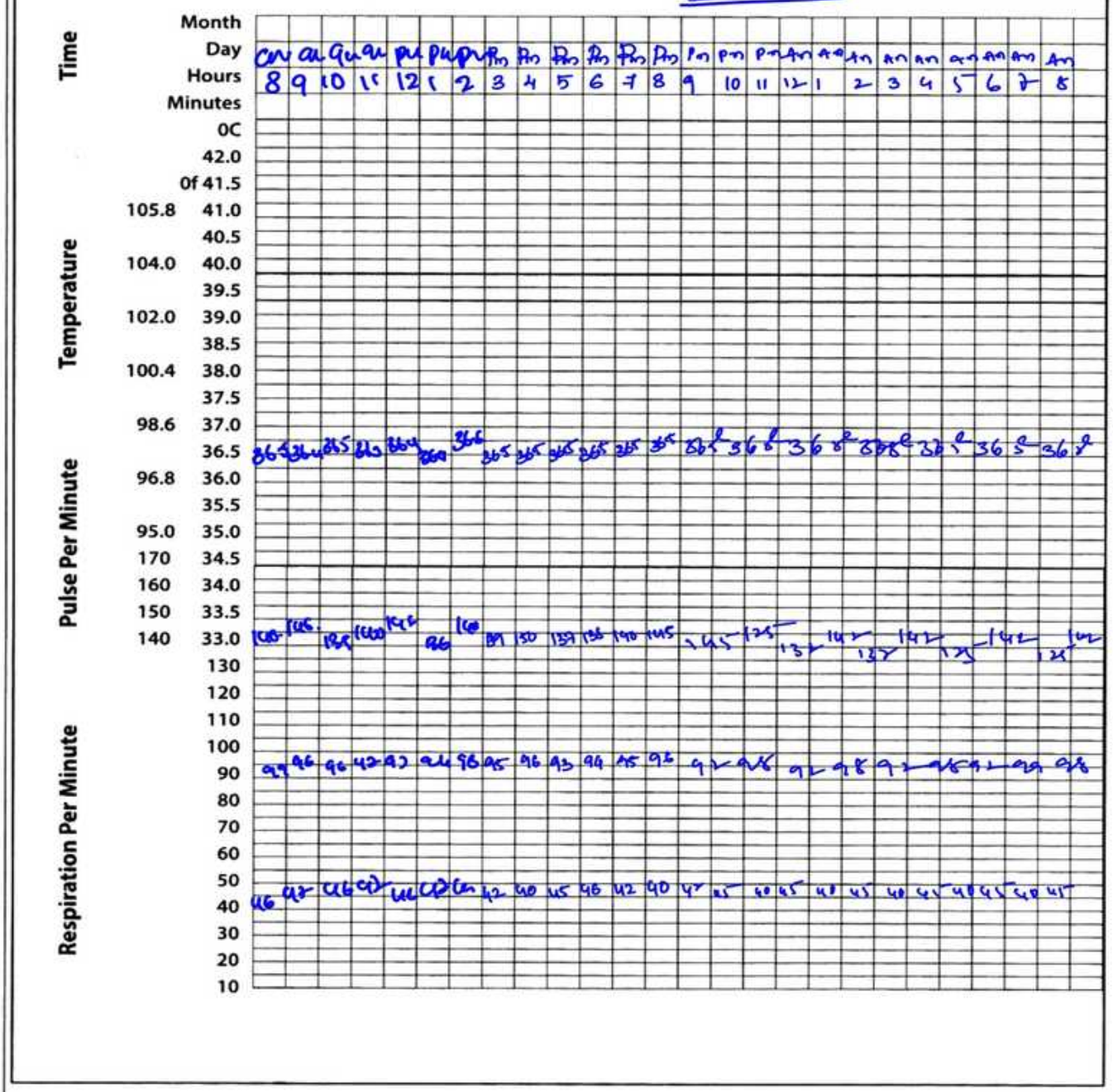
# VITALS MONITORING CHART

ankura hospital for women & children

9M

| | |
|---|---|
| MR No. : | Name : VAGDEVI<br>UHID NO : 162608635<br>IPD NO : LB26/3007 |
| Patient N | |
| Age : | Age : 4 D Gender : Male |
| Sex : | Consult : DR. SRINIVAS MURKI<br>Adm Date : 27-06-2026 09:03 PM |

Current Weight:
Current Height:
Blood Group & Rh Type:
Date : 30/6/26



Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# VITALS MONITORING CHART

ankura hospital for women & children

9M

MR No. :
Patient Na
Age :
Sex :

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Current Weight:
Current Height:
Blood Group & Rh Type:
Date : 01/7/26 8am to 8pm

Time
Month
Day
Hours: AM AM AM PM PM PM PM
Minutes: 8 9 10 11 12 1 2

Temperature
0C
42.0
Of 41.5
105.8 41.0
40.5
104.0 40.0
39.5
102.0 39.0
38.5
100.4 38.0
37.5
98.6 37.0
36.5

Pulse Per Minute
96.8 36.0
35.5
95.0 35.0
170 34.5
160 34.0
150 33.5
140 33.0
130
120
110
100
90
80
70
60
50
40
30
20
10

Respiration Per Minute

Blood Pressure mmHg
250
240
230
220
210
200
190
180
170
160
150
140
130
120
110
100
90
80
70
60
50
40
30
20
10

# PATIENT & FAMILY EDUCATION FORM

ankura hospital for women & children | 9M

PATIENT NAME: Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR NO: _____

DOB/AGE: _____ GENDER: _____ IP NO: _____

| DATE & TIME | EDUCATION/INFORMATION PROVIDED ON (TOPIC CONTENT) | NAME & SIGN OF THE STAFF | NAME & SIGN OF THE PATIENT / ATTENDANT | REMARKS |
|---|---|---|---|---|
| 28/6/26 11 A.M | Baby on minimal $O_2$<br>4-5 episodes of seizures (+) overnight<br>Plan SOS ventilation & inj. midazolam<br>GRBS → 60 mg/dl | | | |
| 29/6/26 11 am | No further seizures<br>stop Levipril<br>Continue Gardenal / Optineuron<br>Neuro (0)<br>MRI brain today | | | |
| 29/6/26 7.30 P.M | no further seizures<br>Stable<br>Plan to do MRI after sugar is normal | | | |

# PATIENT & FAMILY EDUCATION FORM

ankura hospital for women & children

9M By ankura hospital Birth & Beyond

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

PATIENT NAME: _____ MR NO: _____

DOB/AGE: _____ GENDER: _____ IP NO: _____

| DATE & TIME | EDUCATION/INFORMATION PROVIDED ON (TOPIC CONTENT) | NAME & SIGN OF THE STAFF | NAME & SIGN OF THE PATIENT / ATTENDANT | REMARKS |
|---|---|---|---|---|
| 30/6/26 11 A.M | on further seizures (on hypoglycemia stable | TRJ. | Vidi // | |
| 30/6/26 8 PM | NO seizure - Hypoglycemia stable<br>MRI Brain scan<br>awaiting<br>S/O Occipital lobe involvement<br>more on (R) side<br>Reasons has explained | dy | Vidi ✓ | |

# PATIENT & FAMILY EDUCATION FORM

ankura hospital for women & children

9M

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

PATIENT NAM _____ MR NO: _____

DOB/AGE: _____ GENDER: _____ IP NO: _____

| DATE & TIME | EDUCATION/INFORMATION PROVIDED ON (TOPIC CONTENT) | NAME & SIGN OF THE STAFF | NAME & SIGN OF THE PATIENT / ATTENDANT | REMARKS |
|---|---|---|---|---|
| 1/7/26 11 AM | NB fracture reflux<br>Discharge<br>Baby stable | | | |

# PERIPHERAL CANNULATION SHEET

ankura hospital for women & children | 9M

**Patient Name:** Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

**Age/Sex:** **D.O.A:**

**MR.No/IP.No:** **Department:**

**1st CANNULA:**

**Insertion Date:** 27/6/26 **Time:** 11 pm **Inserted by:** Sr. Rafi

**Place of Insertion:** Right Hand

**Cannula size:** 24.4. **Removal Date:** _____ **Removed by:** _____

**Reason:** _____

**2nd CANNULA:**

**Insertion Date:** _____ **Time:** _____ **Inserted by:** _____

**Place of Insertion:** _____

**Cannula size:** _____ **Removal Date:** _____ **Removed by:** _____

**Reason:** _____

**3rd CANNULA:**

**Insertion Date:** _____ **Time:** _____ **Inserted by:** _____

**Place of Insertion:** _____

**Cannula size:** _____ **Removal Date:** _____ **Removed by:** _____

**Reason:** _____

**IF PERIPHARAL VENOUS THERMBOPHLEBITIS:**

**Thrombophlebitis identified on:** _____

**Grade of Thrombophlebitis GRO / GR1 / GR2 / GR3 / GR4 / GR5 (Please Tick)**

**INFLUSATE SOLUTION ON FLOW:**

| Name of the Drug and Mg | Name of the Fluids and Rate of on flow per Hour |
|---|---|
| | |
| | |
| | |
| | |
| | |

Note:
GR0 - No Pain or Redness (observe the cannula)
GR1 - Pain or Redness (observe the cannula)
GR2 - Pain, erythema, swelling (resite the cannula)
GR3 - Pain, erythema, swelling (resite and treatment)
GR4 - Pain, erythema, induration, palpable venous cord (resite and treatment)
GR5 - Pain, erythema, induration, palpable venous cord, pyrexia (resite and treatment)

**Name and sign:** _____ **Date and Time:** _____

# PERIPHERAL CANNULATION SHEET

ankura hospital for women & children

9M

| DATE | TIME | VIP SCORE | REMARKS | SIGNATURE |
|---|---|---|---|---|
| 24/6/26 | 9pm | -0- | " | RJ |
| 25/6/26 | 8am | -0- | — | Sraswya |
| 28/06/26 | 2pm | -0- | — | Shanely. |
| 27/6/26 | 8pm | -0- | — | " |
| 30/6/26 | 8am | -0- | — | |
| 30/6/26 | 8pm | -0- | - | " |
| 1/07/26 | 8am | -0- | | |

# NOSOCOMIAL INFECTION SUREILLANCE FORM

ankura hospital for women & children | 9M

**PATIENT DETAILS:**

| | | | | |
|---|---|---|---|---|
| **Name of the patie** | Name : VAGADAVI<br>UHID NO : 162608635<br>IPD NO : LB26/3007<br>Age : 4 D Gender : Male<br>Consult : DR. SRINIVAS MURKI<br>Adm Date : 27-06-2026 09:03 PM | | **Age:** | **Sex:** |
| **MR no/IP no:** | | **DOA:** | **Ward:** | |
| **Consultant name:** | | | | |

**Signs and symptoms:**

**Diagnosis:**

**Invasive devices Details:**

| | Insertion date | Removal date |
|---|---|---|
| 1. IV cannula | 27/6/26 | |
| | | |
| 2. Foley's catheter | 27/6/26 | |
| | | |
| 3. Central line UVC | 28/6/26 | |
| | | |
| 4. Ventilator | | |
| | | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# REQUISTION FORM
(Diagnostics Services)

ankura hospital for women & children

9M

Patient Name: B/o Vagadevi Age/Sex: male/4 dy MR No: 162608635

Date & Time: 28/6/26 Ref.by Dr.: Drs. Nandini Sample Collected By: Roje

Primary Sample Type: Blood/ Urine/ Sputum/ Pus/ Fluid - Pleural/ Pericardial/ Ascitic/ CSF/ Others: _____

Sample Site: _____ Lab Sample Receiving Date & Time: _____ Ward/Room No.: _____

Clinical Diagnosis & H/O Administration of Drugs: _____

## HEAMOGLOBIN / PATHOLOGY

- O Complete Blood Picture(CBP)
- O PCV
- O Platelet Count
- O Hemoglobin
- O AEC Absolute Eosinophil Count) O Peripheral Smear
- O Smear for MP
- O Parasite V & F
- O QBC
- O ESR (Erythrocyte Sedimentation Rate)
- O Blood Group & Rh Type O Direct Coombs Test
- O Indirect Coombs Test
- O Bleeding Time
- O Clotting Time
- O Complete Urine Examination (CUE)
- O Stool Routine Examination
- O CSF Cell Type & Count
- O Factor Assay
- O Anemia Profile
- O Special Investigations:
  1. CBP CRP. RBS LFT.
  2. Bloodels VBG. Blood Group.
  3. KFT. I. Calcium.

## MICROBIOLOGY

- O Blood For Culture & Sensitivity
- O Urine For Culture & Sensitivity
- O Stool For Culture & Sensitivity
- O Throat Swab For Culture & Sensitivity
- O CSF For Culture & Sensitivity
- O AFB For Culture & Sensitivity
- O CSF Lactate
- O CSF Pyruvate
- O HIV I & II (Rapid)
- O HIVI & II (ELISA)
- O HbsAg
- O Hepatitis - A (IgM)
- O Leptospira IgM
- O Weil Felix
- O HCV
- O Dengue IgG
- O Dengue IgM
- O Dengue Ns1 Antigen
- O TB PCR
- O HSV-PCR
- O Mantoux Test
- O RA Factor
- O Rotavirus Antigen Defection
- O ASO
- O Widal Test
- O TORCH Profile
- O Triple Marker
- O Urine Pregnancy Test (UPT)
- O Gram Stain
- O AFB Smear
- O VDRL
- O Serum IgE
- O New Born Screening
- O Metabolic Screening
- O Karotype Test
- O Syndrome Evaluation System (SES)

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# REQUISTION FORM
(Diagnostics Services)

ankura hospital for women & children

9M

Patient Name: B/o Vagadevi Age/Sex: male/4 dy MR No: 162608635

Date & Time: 28/6/26 Ref.by Dr.: DR S. Nandini Sample Collected By: Reje

Primary Sample Type: Blood/ Urine/ Sputum/ Pus/ Fluid - Pleural/ Pericardial/ Ascitic/ CSF/ Others: _____

Sample Site: _____ Lab Sample Receiving Date & Time: _____ Ward/Room No.: _____

Clinical Diagnosis & H/O Administration of Drugs: _____

## HEAMOGLOBIN / PATHOLOGY

- O Complete Blood Picture(CBP)
- O PCV
- O Platelet Count
- O Hemoglobin
- O AEC Absolute Eosinophil Count) O Peripheral Smear
- O Smear for MP
- O Parasite V & F
- O QBC
- O ESR (Erythrocyte Sedimentation Rate)
- O Blood Group & Rh Type O Direct Coombs Test
- O Indirect Coombs Test
- O Bleeding Time
- O Clotting Time
- O Complete Urine Examination (CUE)
- O Stool Routine Examination
- O CSF Cell Type & Count
- O Factor Assay
- O Anemia Profile
- O Special Investigations:
  1. CUG
  2.
  3.

## MICROBIOLOGY

- O Blood For Culture & Sensitivity
- O Urine For Culture & Sensitivity
- O Stool For Culture & Sensitivity
- O Throat Swab For Culture & Sensitivity
- O CSF For Culture & Sensitivity
- O AFB For Culture & Sensitivity
- O CSF Lactate
- O CSF Pyruvate
- O HIV I & II (Rapid)
- O HIVI & II (ELISA)
- O HbsAg
- O Hepatitis - A (IgM)
- O Leptospira IgM
- O Weil Felix
- O HCV
- O Dengue IgG
- O Dengue IgM
- O Dengue Ns1 Antigen
- O TB PCR
- O HSV-PCR
- O Mantoux Test
- O RA Factor
- O Rotavirus Antigen Defection
- O ASO
- O Widal Test
- O TORCH Profile
- O Triple Marker
- O Urine Pregnancy Test (UPT)
- O Gram Stain
- O AFB Smear
- O VDRL
- O Serum IgE
- O New Born Screening
- O Metabolic Screening
- O Karotype Test
- O Syndrome Evaluation System (SES)

# REQUISTION FORM
(Diagnostics Services)

ankura hospital for women & children

9M

Patient Name: B/o. Vagadevi Age/Sex: D4/male MR No: 162608635

Date & Time: 28/6/26 Ref.by Dr.: Dr. S. Mukti Sample Collected By: Sis. Anaruya

Primary Sample Type: ~~Blood~~/ Urine/ Sputum/ Pus/ Fluid - Pleural/ Pericardial/ Ascitic/ CSF/ Others: _____

Sample Site: Umbilical Lab Sample Receiving Date & Time: _____ Ward/Room No.: NCU

Clinical Diagnosis & H/O Administration of Drugs: _____

## HEAMOGLOBIN / PATHOLOGY

- O Complete Blood Picture(CBP)
- O PCV
- O Platelet Count
- O Hemoglobin
- O AEC Absolute Eosinophil Count) O Peripheral Smear
- O Smear for MP
- O Parasite V & F
- O QBC
- O ESR (Erythrocyte Sedimentation Rate)
- O Blood Group & Rh Type O Direct Coombs Test
- O Indirect Coombs Test
- O Bleeding Time
- O Clotting Time
- O Complete Urine Examination (CUE)
- O Stool Routine Examination
- O CSF Cell Type & Count
- O Factor Assay
- O Anemia Profile
- O Special Investigations:
  1. Ammonia
  2.
  3.

## MICROBIOLOGY

- O Blood For Culture & Sensitivity
- O Urine For Culture & Sensitivity
- O Stool For Culture & Sensitivity
- O Throat Swab For Culture & Sensitivity
- O CSF For Culture & Sensitivity
- O AFB For Culture & Sensitivity
- O CSF Lactate
- O CSF Pyruvate
- O HIV I & II (Rapid)
- O HIVI & II (ELISA)
- O HbsAg
- O Hepatitis - A (IgM)
- O Leptospira IgM
- O Weil Felix
- O HCV
- O Dengue IgG
- O Dengue IgM
- O Dengue Ns1 Antigen
- O TB PCR
- O HSV-PCR
- O Mantoux Test
- O RA Factor
- O Rotavirus Antigen Defection
- O ASO
- O Widal Test
- O TORCH Profile
- O Triple Marker
- O Urine Pregnancy Test (UPT)
- O Gram Stain
- O AFB Smear
- O VDRL
- O Serum IgE
- O New Born Screening
- O Metabolic Screening
- O Karotype Test
- O Syndrome Evaluation System (SES)

# REQUISTION FORM
(Diagnostics Services)

ankura hospital for women & children

9M

Patient Name: B/o. vagadevi Age/Sex: 5days/male MR No: 162608635

Date & Time: 28/6/26 Ref.by Dr.: Dr. S. mukki Sample Collected By: Sis. Anasuya

Primary Sample Type: Blood/ ~~Urine~~/ Sputum/ Pus/ Fluid - Pleural/ Pericardial/ Ascitic/ CSF/ Others: _____

Sample Site: Urine Lab Sample Receiving Date & Time: _____ Ward/Room No.: NICU

Clinical Diagnosis & H/O Administration of Drugs: _____

## HEAMOGLOBIN / PATHOLOGY

- O Complete Blood Picture(CBP)
- O PCV
- O Platelet Count
- O Hemoglobin
- O AEC Absolute Eosinophil Count) O Peripheral Smear
- O Smear for MP
- O Parasite V & F
- O QBC
- O ESR (Erythrocyte Sedimentation Rate)
- O Blood Group & Rh Type O Direct Coombs Test
- O Indirect Coombs Test
- O Bleeding Time
- O Clotting Time
- O Complete Urine Examination (CUE)
- O Stool Routine Examination
- O CSF Cell Type & Count
- O Factor Assay
- O Anemia Profile
- O Special Investigations:
  1. Urine for reducing substance
  2.
  3.

## MICROBIOLOGY

- O Blood For Culture & Sensitivity
- O Urine For Culture & Sensitivity
- O Stool For Culture & Sensitivity
- O Throat Swab For Culture & Sensitivity
- O CSF For Culture & Sensitivity
- O AFB For Culture & Sensitivity
- O CSF Lactate
- O CSF Pyruvate
- O HIV I & II (Rapid)
- O HIVI & II (ELISA)
- O HbsAg
- O Hepatitis - A (IgM)
- O Leptospira IgM
- O Weil Felix
- O HCV
- O Dengue IgG
- O Dengue IgM
- O Dengue Ns1 Antigen
- O TB PCR
- O HSV-PCR
- O Mantoux Test
- O RA Factor
- O Rotavirus Antigen Defection
- O ASO
- O Widal Test
- O TORCH Profile
- O Triple Marker
- O Urine Pregnancy Test (UPT)
- O Gram Stain
- O AFB Smear
- O VDRL
- O Serum IgE
- O New Born Screening
- O Metabolic Screening
- O Karotype Test
- O Syndrome Evaluation System (SES)

# BILLING RECORD

ankura hospital | 9M

| | | | |
|---|---|---|---|
| Patient Name: | B/o Vagadevi | MR. No.: | 162608635 |
| Doctor's Name: | Mrs. Murthy | Ward / Room No. | NFW |
| Age / Gender: | | Date & Time: | 27/6/26 |

| S.No | PROCEDURES | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Arterial Line Insertion | | | |
| 2 | Ascitic Tap | | | |
| 3 | Bone Marrow Aspiration | | | |
| 4 | Central Line Insertion | | | |
| 5 | CPR | | | |
| 6 | CVP line Insertion | | | |
| 7 | Dressing Minor/Medium/Major/Special | | | |
| 8 | Emergency/Elective Intubation | | | |
| 9 | Foleys Catherization | | | ✓ |
| 10 | HD Cathter | | | |
| 11 | Hemodialysis | | | |
| 12 | ICD Insertion | | | |
| 13 | IV Cannulation | | | ✓ |
| 14 | Lumbar Drain insertion | | | |
| 15 | Lumbar Puncture | | | |
| 16 | Tracheostomy | | | |
| 17 | Nebulization | | | |
| 18 | PICC line Insertion | | | |
| 19 | Pigtail Catheter | | | |
| 20 | Plueral Tap | | | |
| 21 | Ryles Tube Insertion | | | |
| 22 | Sedation Anesthesia | | | |
| 23 | Surfactant Administration. | | | |
| 24 | Tracheostomy Decannulation | | | |
| 25 | Tracheostomy Tube Change | | | |
| 26 | Umblical Catheterisation. | | | |
| 27 | Ventricular Tapping | | | |
| 28 | ABG (Arterial Blood Gas Analysis) | | | |
| 29 | GRBS (Blood Glucose by Glucometer). | | | |
| 30 | ECG-12 Lead | | | 5 |
| 31 | Urine Sugar/Ketone (Rato Strips/Albumin) | | | |
| 32 | Blood Transfusions | | | |

| | Others | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| | | | | |
| | | | | |
| | | | | |
| | | | | |
| | | | | |

| S. No. | LAB INVESTIGATIONS, Specify | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | CBP LRP LFT. KFT | | | ✓ |
| 2 | T. Sodium RBS VBG | | | ✓ |
| 3 | CUG, | | | ✓ |
| 4 | | | | |
| 5 | | | | |
| 6 | | | | |

| | RADIOLOGY INVESTIGATIONS | | | |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |

| | CROSS CONSULTATION | | | |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| 5 | | | | |

| | BEDSIDE EQUIPMENTS | Start | Stop | Start | Stop | Start | Stop |
|---|---|---|---|---|---|---|---|
| 1 | Alpha Matress/DVT Pump | | | | | | |
| 2 | Body warmer | | | | | 7AM | |
| 3 | BIPAP | | | | | | |
| 4 | CPAP | | | | | | |
| 5 | NST/CTG/Fetal Monitoring | | | | | | |
| 6 | Infusion/Syringe Pump (2) | | | | | 7AM | |
| 7 | Phototherapy Bilipt | | | | | 10AM | |
| 8 | Oxygen Charges | | | | | 7AM | |
| 9 | Ventillator Charges | | | | | | |
| 10 | Monitor charges | | | | | 7AM | |

| | CONSULTATION | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Doctors Visit | | | ✓ |
| 2 | Physiotherapy | | | |
| 3 | Diet | | | |
| 4 | OT Charges | | | |
| 5 | Transfer IN/OUT | | | |

| | OTHERS | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |

| | | | | |
|---|---|---|---|---|
| Name of Nursing staff | | | Roja | Name of PCC/Billing staff (Recieving Staff) |
| Signature | | | | Signature |
| Employee ID | | | 16636 | Employee ID |

Note: Nurse must update the sheet after each procedure
Nurses must sign (Shift wise) once the details are completely filled in the billing Activity Sheet
One copy of billing activity sheet must be sent to billing department daily before 8.00 am by concerned Nursing staff
Daily updation of patient bills in HIMS by billing staff must be done
Final verification of bill from the file at the time of discharge must be done

| | |
|---|---|
| Name of the Staff Handing Over | |
| Signature | |
| Employee ID | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

# BILLING RECORD

ankura hospital | 9M

| | |
|---|---|
| Patient Name: B/o. vagdevi | MR. No.: 162608635 |
| Doctor's Name: Dr. S. murki SIT | Ward / Room No. NICU |
| Age / Gender: 4 Days / | Date & Time: 28/6/26 |

| S.No | PROCEDURES | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Arterial Line Insertion | | | |
| 2 | Ascitic Tap | | | |
| 3 | Bone Marrow Aspiration | | | |
| 4 | Central Line Insertion | | | |
| 5 | CPR | | | |
| 6 | CVP line Insertion | | | |
| 7 | Dressing Minor/Medium/Major/Special | | | |
| 8 | Emergency/Elective Intubation | | | |
| 9 | Foleys Catherization | | | |
| 10 | HD Cathter | | | |
| 11 | Hemodialysis | | | |
| 12 | ICD Insertion | | | |
| 13 | IV Cannulation | | | |
| 14 | Lumbar Drain insertion | | | |
| 15 | Lumbar Puncture | | | |
| 16 | Tracheostomy | | | |
| 17 | Nebulization | | | |
| 18 | PICC line Insertion | | | |
| 19 | Pigtail Catheter | | | |
| 20 | Plueral Tap | | | |
| 21 | Ryles Tube Insertion | | | |
| 22 | Sedation Anesthesia | | | |
| 23 | Surfactant Administration. | | | |
| 24 | Tracheostomy Decannulation | | | |
| 25 | Tracheostomy Tube Change | | | |
| 26 | Umblical Catheterisation. | ✓ | | |
| 27 | Ventricular Tapping | | | |
| 28 | ABG (Arterial Blood Gas Analysis) | | | |
| 29 | GRBS (Blood Glucose by Glucometer). | ✓ | | ✓✓ |
| 30 | ECG-12 Lead | | | |
| 31 | Urine Sugar/Ketone (Rato Strips/Albumin) | | | |
| 32 | Blood Transfusions | | | |

| | Others | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| | | | | |
| | | | | |
| | | | | |
| | | | | |
| | | | | |

| S. No. | LAB INVESTIGATIONS, Specify | Morning shift |
|---|---|---|
| 1 | Amonia, | ✓ |
| 2 | Urine for R/S | ✓ |
| 3 | EEG | ✓ |
| 4 | | |
| 5 | | |
| 6 | | |
| | RADIOLOGY INVESTIGATIONS | |
| 1 | x-ray. | ✓ |
| 2 | | |
| 3 | | |
| 4 | | |
| | CROSS CONSULTATION | |
| 1 | | |
| 2 | | |
| 3 | | |
| 4 | | |
| 5 | | |

| | BEDSIDE EQUIPMENTS | Start | Stop | Start | Stop | Start | Stop |
|---|---|---|---|---|---|---|---|
| 1 | Alpha Matress/DVT Pump | | | | | | |
| 2 | Body warmer | ✓ | | CNT | | ✓ | |
| 3 | BIPAP | | | | | | |
| 4 | CPAP | | | | | | |
| 5 | NST/CTG/Fetal Monitoring | | | | | | |
| 6 | Infusion/Syringe Pump (2) | ✓ | | CNT | | ✓ | |
| 7 | Phototherapy SSPT | ✓ | | CNT | | ✓ | |
| 8 | Oxygen Charges | ✓ | | CNT | | ✓ | BAD |
| 9 | Ventillator Charges | | | | | | |
| 10 | Monitor charges | ✓ | | CNT | | ✓ | |

| | CONSULTATION | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Doctors Visit | ✓ | ✓ | ✓ |
| 2 | Physiotherapy | | | |
| 3 | Diet | | | |
| 4 | OT Charges | | | |
| 5 | Transfer IN/OUT | | | |

| | OTHERS | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |

| | | | |
|---|---|---|---|
| Name of Nursing staff | Anasuya | Shanty | Rafe |
| Signature | Anup | Sh | |
| Employee ID | 12277 | 2634 | 10636 |

Name of PCC/Billing staff (Recieving Staff)

Signature

Employee ID

Note: Nurse must update the sheet after each procedure
Nurses must sign (Shift wise) once the details are completely filled in the billing Activity Sheet
One copy of billing activity sheet must be sent to billing department daily before 8.00 am by concerned Nursing staff
Daily updation of patient bills in HIMS by billing staff must be done
Final verification of bill from the file at the time of discharge must be done

| |
|---|
| Name of the Staff Handing Over |
| Signature |
| Employee ID |

# BILLING RECORD

ankura hospital | 9M

| | | | |
|---|---|---|---|
| Patient Name: | B/O vagdevi | MR. No.: | 162608635 |
| Doctor's Name: | Dr. S. MURTHY | Ward / Room No. | NICU |
| Age / Gender: | 1 male | Date & Time: | 29/06/26 |

| S.No | PROCEDURES | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Arterial Line Insertion | | | |
| 2 | Ascitic Tap | | | |
| 3 | Bone Marrow Aspiration | | | |
| 4 | Central Line Insertion | | | |
| 5 | CPR | | | |
| 6 | CVP line Insertion | | | |
| 7 | Dressing Minor/Medium/Major/Special | | | |
| 8 | Emergency/Elective Intubation | | | |
| 9 | Foleys Catherization | | | |
| 10 | HD Cathter | | | |
| 11 | Hemodialysis | | | |
| 12 | ICD Insertion | | | |
| 13 | IV Cannulation | | | |
| 14 | Lumbar Drain insertion | | | |
| 15 | Lumbar Puncture | | | |
| 16 | Tracheostomy | | | |
| 17 | Nebulization | | | |
| 18 | PICC line Insertion | | | |
| 19 | Pigtail Catheter | | | |
| 20 | Plueral Tap | | | |
| 21 | Ryles Tube Insertion | | | |
| 22 | Sedation Anesthesia | | | |
| 23 | Surfactant Administration. | | | |
| 24 | Tracheostomy Decannulation | | | |
| 25 | Tracheostomy Tube Change | | | |
| 26 | Umblical Catheterisation. | | | |
| 27 | Ventricular Tapping | | | |
| 28 | ABG (Arterial Blood Gas Analysis) | | | |
| 29 | GRBS (Blood Glucose by Glucometer). | | ✓ | ✓✓ |
| 30 | ECG-12 Lead | | | |
| 31 | Urine Sugar/Ketone (Rato Strips/Albumin) | | | |
| 32 | Blood Transfusions | | | |

| | Others | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| | | | | |
| | | | | |
| | | | | |
| | | | | |
| | | | | |

| S. No. | LAB INVESTIGATIONS, Specify | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| 5 | | | | |
| 6 | | | | |
| | RADIOLOGY INVESTIGATIONS | | | |
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| | CROSS CONSULTATION | | | |
| 1 | Neurologist opinion done | | ✓ | |
| 2 | | | | |
| 3 | NSG done. | | ✓ | |
| 4 | | | | |
| 5 | | | | |

| | BEDSIDE EQUIPMENTS | Start | Stop | Start | Stop | Start | Stop |
|---|---|---|---|---|---|---|---|
| 1 | Alpha Matress/DVT Pump | | | | | | |
| 2 | Body warmer | CNT | | ✓ | | ✓ | |
| 3 | BIPAP | | | | | | |
| 4 | CPAP | | | | | | |
| 5 | NST/CTG/Fetal Monitoring | | | | | | |
| 6 | Infusion/Syringe Pump (2) | CNT | | ✓ | | ✓ | |
| 7 | Phototherapy 8/3/p | ✓ | | 2pm | | | |
| 8 | Oxygen Charges | | | | | | |
| 9 | Ventillator Charges | | | | | | |
| 10 | Monitor charges | CNT | | ✓ | | ✓ | |

| | CONSULTATION | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Doctors Visit | ✓ | ✓ | ✓ |
| 2 | Physiotherapy | | | |
| 3 | Diet | | | |
| 4 | OT Charges | | | |
| 5 | Transfer IN/OUT | | | |

| | OTHERS | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |

| | | | |
|---|---|---|---|
| Name of Nursing staff | Shanty | | Name of PCC/Billing staff (Recieving Staff) |
| Signature | | | Signature |
| Employee ID | 2634 | 2165 | 16536 Employee ID |

Note: Nurse must update the sheet after each procedure
Nurses must sign (Shift wise) once the details are completely filled in the billing Activity Sheet
One copy of billing activity sheet must be sent to billing department daily before 8.00 am by concerned Nursing staff
Daily updation of patient bills in HIMS by billing staff must be done
Final verification of bill from the file at the time of discharge must be done

| | |
|---|---|
| Name of the Staff Handing Over | |
| Signature | |
| Employee ID | |

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

MR. No.: 05635
Ward / Room No. PICU
Date & Time: 30/6/26 8am to 8pm

| S.No | PROCEDURES | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Arterial Line Insertion | | | |
| 2 | Ascitic Tap | | | |
| 3 | Bone Marrow Aspiration | | | |
| 4 | Central Line Insertion | | | |
| 5 | CPR | | | |
| 6 | CVP line Insertion | | | |
| 7 | Dressing Minor/Medium/Major/Special | | | |
| 8 | Emergency/Elective Intubation | | | |
| 9 | Foleys Catherization | | | |
| 10 | HD Cathter | | | |
| 11 | Hemodialysis | | | |
| 12 | ICD Insertion | | | |
| 13 | IV Cannulation | | | |
| 14 | Lumbar Drain insertion | | | |
| 15 | Lumbar Puncture | | | |
| 16 | Tracheostomy | | | |
| 17 | Nebulization | | | |
| 18 | PICC line Insertion | | | |
| 19 | Pigtail Catheter | | | |
| 20 | Plueral Tap | | | |
| 21 | Ryles Tube Insertion | | | |
| 22 | Sedation Anesthesia | | | |
| 23 | Surfactant Administration. | | | |
| 24 | Tracheostomy Decannulation | | | |
| 25 | Tracheostomy Tube Change | | | |
| 26 | Umblical Catheterisation. | | | |
| 27 | Ventricular Tapping | | | |
| 28 | ABG (Arterial Blood Gas Analysis) | | | |
| 29 | GRBS (Blood Glucose by Glucometer). | ✓ | | ✓ |
| 30 | ECG-12 Lead | | | |
| 31 | Urine Sugar/Ketone (Rato Strips/Albumin) | | | |
| 32 | Blood Transfusions | | | |
| | **Others** | Morning shift | Evening Shift | Night Shift |
| | | | | |
| | | | | |
| | | | | |
| | | | | |

| S. No. | LAB INVESTIGATIONS, Specify | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| 5 | | | | |
| 6 | | | | |
| | RADIOLOGY INVESTIGATIONS | | | |
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| | CROSS CONSULTATION | | | |
| 1 | MRI done | | ✓ | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| 5 | | | | |

| | BEDSIDE EQUIPMENTS | Start | Stop | Start | Stop | Start | Stop |
|---|---|---|---|---|---|---|---|
| 1 | Alpha Matress/DVT Pump | | | | | | |
| 2 | Body warmer | ✓ | | CNT | | ✓ | |
| 3 | BIPAP | | | | | | |
| 4 | CPAP | | | | | | |
| 5 | NST/CTG/Fetal Monitoring | | | | | | |
| 6 | Infusion/Syringe Pump (2) | ✓ | | CNT | | ✓ | |
| 7 | Phototherapy | | | | | | |
| 8 | Oxygen Charges | | | | | | |
| 9 | Ventillator Charges | | | | | | |
| 10 | Monitor charges | ✓ | | CNT | | ✓ | |

| | CONSULTATION | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Doctors Visit | ✓ | ✓ | ✓ |
| 2 | Physiotherapy | | | |
| 3 | Diet | | | |
| 4 | OT Charges | | | |
| 5 | Transfer IN/OUT | | | |

| | OTHERS | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | One Syring Pump at 10pm Stop | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |

| | | | |
|---|---|---|---|
| Name of Nursing staff | Rekha | Shancy | Reju |
| Signature | | | |
| Employee ID | 2461 | 2634 | 10636 |

Name of PCC/Billing staff (Recieving Staff)
Signature
Employee ID

Note: Nurse must update the sheet after each procedure
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One copy of billing activity sheet must be sent to billing department daily before 8.00 am by concerned Nursing staff
Daily updation of patient bills in HIMS by billing staff must be done
Final verification of bill from the file at the time of discharge must be done

| Name of the Staff Handing Over |
|---|
| Signature |
| Employee ID |

# BILL

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Patient Na
Doctor's N
Age / Gen

MR. No.: 08635
Ward / Room No. ONCU
Date & Time: 1/07/26 8am to 8am

ankura hospital | 9M

| S.No | | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Arterial Line Insertion | | | |
| 2 | Ascitic Tap | | | |
| 3 | Bone Marrow Aspiration | | | |
| 4 | Central Line Insertion | | | |
| 5 | CPR | | | |
| 6 | CVP line Insertion | | | |
| 7 | Dressing Minor/Medium/Major/Special | | | |
| 8 | Emergency/Elective Intubation | | | |
| 9 | Foleys Catherization | | | |
| 10 | HD Cathter | | | |
| 11 | Hemodialysis | | | |
| 12 | ICD Insertion | | | |
| 13 | IV Cannulation | | | |
| 14 | Lumbar Drain insertion | | | |
| 15 | Lumbar Puncture | | | |
| 16 | Tracheostomy | | | |
| 17 | Nebulization | | | |
| 18 | PICC line Insertion | | | |
| 19 | Pigtail Catheter | | | |
| 20 | Plueral Tap | | | |
| 21 | Ryles Tube Insertion | | | |
| 22 | Sedation Anesthesia | | | |
| 23 | Surfactant Administration. | | | |
| 24 | Tracheostomy Decannulation | | | |
| 25 | Tracheostomy Tube Change | | | |
| 26 | Umblical Catheterisation. | | | |
| 27 | Ventricular Tapping | | | |
| 28 | ABG (Arterial Blood Gas Analysis) | | | |
| 29 | GRBS (Blood Glucose by Glucometer). | | | |
| 30 | ECG-12 Lead | | | |
| 31 | Urine Sugar/Ketone (Rato Strips/Albumin) | | | |
| 32 | Blood Transfusions | | | |
| | **Others** | Morning shift | Evening Shift | Night Shift |
| | | | | |
| | | | | |
| | | | | |
| | | | | |
| | | | | |

| S. No. | LAB INVESTIGATIONS, Specify | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| 5 | | | | |
| 6 | | | | |
| | RADIOLOGY INVESTIGATIONS | | | |
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| | CROSS CONSULTATION | | | |
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |
| 5 | | | | |

| | BEDSIDE EQUIPMENTS | Start | Stop | Start | Stop | Start | Stop |
|---|---|---|---|---|---|---|---|
| 1 | Alpha Matress/DVT Pump | | | | | | |
| 2 | Body warmer | ✓ | | | | | |
| 3 | BIPAP | | | | | | |
| 4 | CPAP | | | | | | |
| 5 | NST/CTG/Fetal Monitoring | | | | | | |
| 6 | Infusion/Syringe Pump (2) | ✓ | | | | | |
| 7 | Phototherapy | | | | | | |
| 8 | Oxygen Charges | | | | | | |
| 9 | Ventillator Charges | | | | | | |
| 10 | Monitor charges | ✓ | | | | | |

| | CONSULTATION | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | Doctors Visit | ✓ | | |
| 2 | Physiotherapy | | | |
| 3 | Diet | | | |
| 4 | OT Charges | | | |
| 5 | Transfer IN/OUT | | | |

| | OTHERS | Morning shift | Evening Shift | Night Shift |
|---|---|---|---|---|
| 1 | | | | |
| 2 | | | | |
| 3 | | | | |
| 4 | | | | |

| | | | |
|---|---|---|---|
| Name of Nursing staff | Jyothi | Name of PCC/Billing staff (Recieving Staff) | |
| Signature | | Signature | |
| Employee ID | 8465 | Employee ID | |

Note: Nurse must update the sheet after each procedure
Nurses must sign (Shift wise) once the details are completely filled in the billing Activity Sheet
One copy of billing activity sheet must be sent to billing department daily before 8.00 am by concerned Nursing staff
Daily updation of patient bills in HIMS by billing staff must be done
Final verification of bill from the file at the time of discharge must be done

| | |
|---|---|
| Name of the Staff Handing Over | |
| Signature | |
| Employee ID | |

ankura hospital | 9M | 9M

## REQUISITION FOR MICROBIOLOGY & IMMUNO-SEROLOGY

Patient's Name: B/O Vagdevi  Age: 4D  Sex: male

UHID NO: 162608635  Date: 28/6/26  Collection Time:

IP NO/OP NO: LB26/3007  Nature of Specimen: Blood Cls

CLINICAL DIAGNOSIS: Ketoacidosis (SGA) + Hypoglycemia seizures) RSepsis)

Previous Culture No/Date:

Clinical details (Brief history) required with Antibiotic history

Physical findings:

Other pertinent/specialized investigations

Risk Stratification of patient (s)

| Patient Type-1 (CAI) | Patient Type-2 (HAI) | Patient Type-3 (NI) |
|---|---|---|
| No contact with health care system | Contact with health care system (e.g. recent hospital admission, nursing home, dialysis) without invasive procedure | Long hospitalization and or invasive procedures |
| No prior antibiotic treatment | Recent antibiotic therapy | Recent & multiple antibiotic therapies |
| Patient young with few co-morbid conditions | Patient old with multiple co-morbidities | Patient characteristics: cystic fibrosis, structural lung disease, advanced AIDS, neutropenia, other severe immunodeficiency. |

Signature of the Doctor

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9M by ankura hospital

# GRBS MONITORING

Patient Name: B/o Vagdevi Age: 4 dy Sex: male IP No: 8635

Consultant: DOA: Diagnosis

| Date | Time | GRBS | Informed to (Dr Name | Nurse Sign | Parent Sign |
|---|---|---|---|---|---|
| 27/6 | 9:30pm | 36 mg dl | Dr. Lavanya | | |
| | 10pm | 65 mg dl | Dr Lavanya | | |
| | 11pm | 72 mg dl | Dr Lavanya | Koje | |
| 28/6 | 2AM | 75 mg dl | Dr Lavanya | | |
| | 5AM | 105 mg dl | Dr Lavanya | | |
| 28/6 | 1pm | 93 mg/dl | Dr. Ananthsri | Anaveya | |
| 28/6 | 11pm | 50 mg dl | Dr Ananthsri | Koje | |
| 29/6 | 6AM | 98 mg dl | Dr Ananthsri | Koje | |
| 29/6 | 6pm | 122 mg/dl | Dr Tejaswi | | |
| 29/6 | 4pm | 99 mg dl | Dr. Tejaswi | | |
| 30/6 | 4AM | 109 mg dl | Dr Tejaswi | | |
| 30/6 | 10am | 105 mg/dl | Dr-Tejaswi | | |
| 30/6 | 12AM | 99 mg dl | Dr-Ananthsri | | |
| 1/7 | 5AM | 94 mg dl | Dr Ananthsri | | |
| | | | | | |
| | | | | | |
| | | | | | |
| | | | | | |
| | | | | | |
| | | | | | |

Name : VAGADAVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

IRD No_____

## ANKURA HOSPITAL FOR WOMEN AND CHILDREN - PATIENT ACUITY SCORE & ALLOCATION

| | | SCORE | DATE | MORNING SCORE | STAFF NAME | LEVEL | AFTER NOON SCORE | STAFF NAME | LEVEL | NIGHT SCORE | STAFF NAME | LEVEL |
|---|---|---|---|---|---|---|---|---|---|---|---|---|
| | | | 28/6/26 | | | | | | | | | |
| LEVEL OF CONSCIOUSNESS | Alert age appropriate | 0 | | 0 | Anasuya | | | | | | | |
| | Irritable or lethargic | 1 | | | | | 01 | Shanty | | 01 | Loje | |
| | Responds to pain | 2 | | | | | | | | | | |
| | Unconscious / seizures | 3 | | | | | | | | | | |
| RESPIRATORY STATUS | Normal breathing room air | 0 | | | | | | | | 01 | Loje | |
| | Mild distress oxygen support ✓ | 1 | | 1 | Anasuya | | 01 | Shanty | | | | |
| | Moderate distress / frequent suction | 2 | | | | | | | | | | |
| | Severe distress CPAP / Ventilator | 3 | | | | | | | | | | |
| CARDIOVASCULAR STATUS | Stable vitals | 0 | | | | | | | | 01 | Loje | |
| | Tachycardia / IV fluids ✓ | 1 | | 1 | Anasuya | | 01 | Shanty | | | | |
| | Inotropes / invasive monitoring | 2 | | | | | | | | | | |
| | Shock risk | 3 | | | | | | | | | | |
| FEEDING & NUTRITION | Oral feeds independently | 0 | | | | | | | | | | |
| | Assisted feeding | 1 | | | | | | | | 02 | Loje | |
| | NG feeds | 2 | | 2 | Anasuya | | 2 | Shanty | Level 1-2 | | | |
| | TPN / feeding intolerance | 3 | | | | | | | | | | |
| ELIMINATION | Normal urine stool | 0 | | | | | | | | | | |
| | Diaper care assistance | 1 | | | | | | | | | | |
| | Catheter / strict monitoring ✓ | 2 | | 2 | Anasuya | | 02 | Shanty | | 02 | Loje | |
| | Oliguria / anuria | 3 | | | | | | | | | | |
| MEDICATION COMPLEXITY | Oral medications | 0 | | | | | | | | | | |
| | IV medications | 1 | | 1 | Anasuya | | 01 | Shanty | | 01 | Loje | |
| | Multiple IV infusions | 2 | | | | | | | | | | |
| | High risk drug titration | 3 | | | | | | | | | | |
| PAIN & COMFORT | Comfortable | 0 | | | | | | | | | | |
| | Intermittent pain | 1 | | 1 | Anasuya | | 01 | Shanty | | 01 | Loje | |
| | Persistent pain | 2 | | | | | | | | | | |
| | Severe pain continuous care | 3 | | | | | | | | | | |
| PROCEDURES & DEVICES | None | 0 | | | | | | | | | | |
| | IV line | 1 | | 1 | Anasuya | | 01 | Shanty | | 01 | Loje | |
| | Multiple lines / drains | 2 | | | | | | | | | | |
| | Ventilator / central lines | 3 | | | | | | | | | | |
| PARENTAL SUPPORT NEED | Routine guidance | 0 | | | | | | | | | | |
| | Frequent reassuring | 1 | | | | | | | | | | |
| | Anxious Parents | 2 | | 2 | Anasuya | | 02 | Shanty | | 02 | Loje | |
| | Continuous emotional support | 3 | | | | | | | | | | |

| Category I | Category | Category III | Category IV | INCHARGE NAME & SIGN |
|---|---|---|---|---|
| Score 0-6 | II Score 7-19 | Score 20-30 | Score > 30 | |
| Routine nursing care | Increased nursing observation | Close monitoring and high nursing | One-to-one intensive nursing care | NUR.SUPERVISOR NAME & SIGN |
| LEVEL 1 NURSE | LEVEL 2 NURSE | LEVEL 3 NURSE | LEVEL 4 NURSE | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

Name B/o Vagdevi MRD No 08635

DOA 27/06/26

DIAGNOSIS Seizures

## ANKURA HOSPITAL FOR WOMEN AND CHILDREN - PATIENT ACUITY SCORE & ALLOCATION

| | | SCORE | DATE | MORNING SCORE | STAFF NAME | LEVEL | AFTER NOON SCORE | STAFF NAME | LEVEL | NIGHT SCORE | STAFF NAME | LEVEL |
|---|---|---|---|---|---|---|---|---|---|---|---|---|
| | | | 29/06/26 | | | | | | | | | |
| LEVEL OF CONSCIOUSNESS | Alert age appropriate | 0 | | 0 | Shancy | | 0 | | | 0 | | |
| | Irritable or lethargic | 1 | | | | | | | | | | |
| | Responds to pain | 2 | | | | | | | | | | |
| | Unconscious / seizures | 3 | | | | | | | | 0 | | |
| RESPIRATORY STATUS | Normal breathing room air | 0 | | 0 | Shancy | | 0 | | | | | |
| | Mild distress oxygen support | 1 | | | | | | | | | | |
| | Moderate distress / frequent suction | 2 | | | | | | | | | | |
| | Severe distress CPAP / Ventilator | 3 | | | | | | | | | | |
| CARDIOVASCULAR STATUS | Stable vitals | 0 | | 0 | Shancy | | 0 | | | 0 | | |
| | Tachycardia / IV fluids | 1 | | | | | | | | | | |
| | Inotropes / invasive monitoring | 2 | | | | | | | | | | |
| | Shock risk | 3 | | | | | | | | | | |
| FEEDING & NUTRITION | Oral feeds independently | 0 | | | | | | | | | | |
| | Assisted feeding | 1 | | | | | | | | | | |
| | NG feeds | 2 | | 02 | Shancy | Level 1 | 02 | | | 02 | | |
| | TPN / feeding intolerance | 3 | | | | | | | | | | |
| ELIMINATION | Normal urine stool | 0 | | 0 | Shancy | | 0 | | | 0 | | |
| | Diaper care assistance | 1 | | | | | | | | | | |
| | Catheter / strict monitoring | 2 | | | | | | | | | | |
| | Oliguria / anuria | 3 | | | | | | | | | | |
| MEDICATION COMPLEXITY | Oral medications | 0 | | | | | | | | | | |
| | IV medications | 1 | | 01 | Shancy | | 01 | | | 01 | | |
| | Multiple IV infusions | 2 | | | | | | | | | | |
| | High risk drug titration | 3 | | | | | | | | | | |
| PAIN & COMFORT | Comfortable | 0 | | | | | | | | | | |
| | Intermittent pain | 1 | | 01 | Shancy | | 01 | | | 01 | | |
| | Persistent pain | 2 | | | | | | | | | | |
| | Severe pain continuous care | 3 | | | | | | | | | | |
| PROCEDURES & DEVICES | None | 0 | | | | | | | | | | |
| | IV line | 1 | | 01 | Shancy | | 01 | | | 01 | | |
| | Multiple lines / drains | 2 | | | | | | | | | | |
| | Ventilator / central lines | 3 | | | | | | | | | | |
| PARENTAL SUPPORT NEED | Routine guidance | 0 | | | | | | | | | | |
| | Frequent reassuring | 1 | | 01 | Shancy | | 01 | | | 01 | | |
| | Anxious Parents | 2 | | | | | | | | | | |
| | Continuous emotional support | 3 | | | | | | | | | | |

| Category I | Category | Category III | Category IV | INCHARGE NAME & SIGN |
|---|---|---|---|---|
| Score 0-6 | II Score 7-19 | Score 20-30 | Score > 30 | |
| Routine nursing care | Increased nursing observation | Close monitoring and high nursing | One-to-one intensive nursing care | NUR.SUPERVISOR NAME & SIGN |
| LEVEL 1 NURSE | LEVEL 2 NURSE | LEVEL 3 NURSE | LEVEL 4 NURSE | |

Patient Name : B/O. VAGDEVI UHID : 162608635 IPD : LB26/3007 Gender : Male Age : 4 D

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Nam
DOA
DIAGNOSIS

## ANKURA HOSPITAL FOR WOMEN AND CHILDREN - PATIENT ACUITY SCORE & ALLOCATION

| | | SCORE | DATE | MORNING SCORE | STAFF NAME | LEVEL | AFTER NOON SCORE | STAFF NAME | LEVEL | NIGHT SCORE | STAFF NAME | LEVEL |
|---|---|---|---|---|---|---|---|---|---|---|---|---|
| | | | 30/6/26 | | | | | | | | | |
| LEVEL OF CONSCIOUSNESS | Alert age appropriate | 0 | | 0 | | | 0 | Shancy | | 0 | | |
| | Irritable or lethargic | 1 | | | | | | | | | | |
| | Responds to pain | 2 | | | | | | | | | | |
| | Unconscious / seizures | 3 | | | | | | | | | | |
| RESPIRATORY STATUS | Normal breathing room air | 0 | | 0 | | | 0 | Shancy | | 0 | | |
| | Mild distress oxygen support | 1 | | | | | | | | | | |
| | Moderate distress / frequent suction | 2 | | | | | | | | | | |
| | Severe distress CPAP / Ventilator | 3 | | | | | | | | | | |
| CARDIOVASCULAR STATUS | Stable vitals | 0 | | 0 | | | 0 | Shancy | | 0 | | |
| | Tachycardia / IV fluids | 1 | | | | | | | | | | |
| | Inotropes / invasive monitoring | 2 | | | | | | | | | | |
| | Shock risk | 3 | | | | | | | | | | |
| FEEDING & NUTRITION | Oral feeds independently | 0 | | | | | | | | | | |
| | Assisted feeding | 1 | | | | | | | | | | |
| | NG feeds | 2 | | 02 | | | 02 | Shancy | | 02 | | |
| | TPN / feeding intolerance | 3 | | | | | | | | | | |
| ELIMINATION | Normal urine stool | 0 | | 0 | | | 0 | Shancy | | 0 | | |
| | Diaper care assistance | 1 | | | | | | | | | | |
| | Catheter / strict monitoring | 2 | | | | | | | | | | |
| | Oliguria / anuria | 3 | | | | | | | | | | |
| MEDICATION COMPLEXITY | Oral medications | 0 | | | | | | | | | | |
| | IV medications | 1 | | 01 | | | 01 | Shancy | | 01 | | |
| | Multiple IV infusions | 2 | | | | | | | | | | |
| | High risk drug titration | 3 | | | | | | | | | | |
| PAIN & COMFORT | Comfortable | 0 | | | | | | | | | | |
| | Intermittent pain | 1 | | 01 | | | 01 | Shancy | | 01 | | |
| | Persistent pain | 2 | | | | | | | | | | |
| | Severe pain continuous care | 3 | | | | | | | | | | |
| PROCEDURES & DEVICES | None | 0 | | | | | | | | | | |
| | IV line | 1 | | 01 | | | 01 | Shancy | | 01 | | |
| | Multiple lines / drains | 2 | | | | | | | | | | |
| | Ventilator / central lines | 3 | | | | | | | | | | |
| PARENTAL SUPPORT NEED | Routine guidance | 0 | | | | | | | | | | |
| | Frequent reassuring | 1 | | 01 | | | 01 | Shancy | | 01 | | |
| | Anxious Parents | 2 | | | | | | | | | | |
| | Continuous emotional support | 3 | | | | | | | | | | |

| Category I | Category | Category III | Category IV | INCHARGE NAME & SIGN |
|---|---|---|---|---|
| Score 0-6 | II Score 7-19 | Score 20-30 | Score > 30 | |
| Routine nursing care | Increased nursing observation | Close monitoring and high nursing | One-to-one intensive nursing care | NUR.SUPERVISOR NAME & SIGN |
| LEVEL 1 NURSE | LEVEL 2 NURSE | LEVEL 3 NURSE | LEVEL 4 NURSE | |

Name : VAGDEVI
UHID NO : 162608635
IPD NO : LB26/3007
Age : 4 D Gender : Male
Consult : DR. SRINIVAS MURKI
Adm Date : 27-06-2026 09:03 PM

Nam
DOA
DIA

## ANKURA HOSPITAL FOR WOMEN AND CHILDREN - PATIENT ACUITY SCORE & ALLOCATION

| | | SCORE | DATE | MORNING SCORE | STAFF NAME | LEVEL | AFTER NOON SCORE | STAFF NAME | LEVEL | NIGHT SCORE | STAFF NAME | LEVEL |
|---|---|---|---|---|---|---|---|---|---|---|---|---|
| | | | 1/7/26 | | | | | | | | | |
| LEVEL OF CONSCIOUSNESS | Alert age appropriate | 0 | | 0 | | | | | | | | |
| | Irritable or lethargic | 1 | | | | | | | | | | |
| | Responds to pain | 2 | | | | | | | | | | |
| | Unconscious / seizures | 3 | | | | | | | | | | |
| RESPIRATORY STATUS | Normal breathing room air | 0 | | 0 | | | | | | | | |
| | Mild distress oxygen support | 1 | | | | | | | | | | |
| | Moderate distress / frequent suction | 2 | | | | | | | | | | |
| | Severe distress CPAP / Ventilator | 3 | | | | | | | | | | |
| CARDIOVASCULAR STATUS | Stable vitals | 0 | | 0 | | | | | | | | |
| | Tachycardia / IV fluids | 1 | | | | | | | | | | |
| | Inotropes / invasive monitoring | 2 | | | | | | | | | | |
| | Shock risk | 3 | | | | | | | | | | |
| FEEDING & NUTRITION | Oral feeds independently | 0 | | | | | | | | | | |
| | Assisted feeding | 1 | | | | | | | | | | |
| | NG feeds | 2 | | 02 | | | | | | | | |
| | TPN / feeding intolerance | 3 | | | | | | | | | | |
| ELIMINATION | Normal urine stool | 0 | | 0 | | | | | | | | |
| | Diaper care assistance | 1 | | | | | | | | | | |
| | Catheter / strict monitoring | 2 | | | | | | | | | | |
| | Oliguria / anuria | 3 | | | | | | | | | | |
| MEDICATION COMPLEXITY | Oral medications | 0 | | | | | | | | | | |
| | IV medications | 1 | | 01 | | | | | | | | |
| | Multiple IV infusions | 2 | | | | | | | | | | |
| | High risk drug titration | 3 | | | | | | | | | | |
| PAIN & COMFORT | Comfortable | 0 | | | | | | | | | | |
| | Intermittent pain | 1 | | 01 | | | | | | | | |
| | Persistent pain | 2 | | | | | | | | | | |
| | Severe pain continuous care | 3 | | | | | | | | | | |
| PROCEDURES & DEVICES | None | 0 | | | | | | | | | | |
| | IV line | 1 | | 01 | | | | | | | | |
| | Multiple lines / drains | 2 | | | | | | | | | | |
| | Ventilator / central lines | 3 | | | | | | | | | | |
| PARENTAL SUPPORT NEED | Routine guidance | 0 | | | | | | | | | | |
| | Frequent reassuring | 1 | | 01 | | | | | | | | |
| | Anxious Parents | 2 | | | | | | | | | | |
| | Continuous emotional support | 3 | | | | | | | | | | |

| Category I | Category | Category III | Category IV | INCHARGE NAME & SIGN |
|---|---|---|---|---|
| Score 0-6 | II Score 7-19 | Score 20-30 | Score > 30 | |
| Routine nursing care | Increased nursing observation | Close monitoring and high nursing | One-to-one intensive nursing care | NUR.SUPERVISOR NAME & SIGN |
| LEVEL 1 NURSE | LEVEL 2 NURSE | LEVEL 3 NURSE | LEVEL 4 NURSE | |

Ankura Hospitals LB Nagar Pvt Ltd

Sy. No. 9, 11, 12, Plot No. 10, 1/J, opp. UNLIMITED, Saroornagar, Doctors Colony, L. B. Nagar, Hyderabad,Telangana, 500035

Phone : 9053108108 Website : https://www.ankurahospitals.com email : info@ankurahospital.com

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646870 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 10:35PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## BIOCHEMISTRY

### C-REACTIVE PROTEIN (CRP)

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| C-Reactive Protein (CRP) | 2.4 | mg/L | 0.01 - 6.00 | Immunoturbidimetry |

Interpretation :
---
- In many cases the changes in plasma CRP level precede changes in the clinical symptoms.
- The degree of elevation of CRP reflects the mass or activity of the inflamed tissue and in acute inflammation or infection correlates well with disease activity.
- Because the increase is non-specific, it cannot be interpreted without a complete clinical history, and even then, only by comparison with previous values.
- A persistently raised CRP level generally indicates that therapy is ineffective.
- Normal CRP levels do not exclude the presence of minor degrees of acute, localized inflammation or some chronic diseases such as SLE and ulcerative colitis.

--End Of Report--

**Dr. B Roshini Sumalini**
**MBBS,MD PATHOLOGY**
**Consultant Pathologist**

Ankura Hospitals LB Nagar Pvt Ltd

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Phone : 9053108108 Website : https://www.ankurahospitals.com email : info@ankurahospital.com

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646870 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 10:35PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## BIOCHEMISTRY

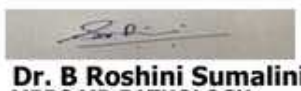
### RENAL FUNCTION TEST (RFT)

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| CREATININE - SERUM | 0.4 | mg/dL | 0.3 - 0.9 | Jaffe Kinetic |
| BLOOD UREA | 20 | mg/dL | 16.60 - 48.50 | Urease |
| BLOOD UREA NITROGEN(BUN) | 9.4 | mg/dL | 6.00 - 20.00 | Calculated |
| URIC ACID | 4.3 | mg/dL | 3.50 - 7.20 | Enzymatic,Colorimetric |
| CALCIUM SERUM | **8.6 L** | mg/dL | 8.80 - 10.80 | Photometric |
| SODIUM SERUM | 142 | mmol/L | 133.00 - 144.00 | |
| POTASSIUM SERUM | 5.1 | mmol/L | 3.40 - 5.10 | IFCC |
| CHLORIDE SERUM | 107 | mmol/L | 97.00 - 107.00 | |

**Interpretation :**

A kidney functiontest is a collective term for a variety of individual tests and procedures thatcan be done to evaluate how well the kidney is functioning. Many conditions canaffect the ability of the kidneys to carry out their vital functions. Some leadto a rapid (Acute) decline in kidney function others lead to a gradual (Chronic)decline in function. Both result in a buildup of toxic waste subset done onurine samples. As well as on blood samples. A number of symptoms may indicate aproblem with yours kidneys. These include high blood pressure, blood in urinefrequent urges to urinate, difficulty beginning urination, painful urination,swelling in the hands and feet due to a buildup of fluids in the body. A singlesymptom may not mean something serious. However, when occurring simultaneously,these symptoms suggest that your kidneys are not working properly kidneyfunction tests can help determine the reason Electrolytes are present in thehuman body and the balancing act of the Electrolytes in our bodies is essentialfor normal function of our cells and organs. There has to be a balance Ionizedcalcium this test if you have signs of kidney or Parathyroid disease. The testmay also be done to monitor progress and treatment of these diseases.

--End Of Report--



**Dr. B Roshini Sumalini**
**MBBS,MD PATHOLOGY**
**Consultant Pathologist**

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Phone : 9053108108 Website : https://www.ankurahospitals.com email : info@ankurahospital.com

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646870 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 10:35PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## HEMATOLOGY

### COMPLETE BLOOD PICTURE (CBP)/CBC

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| HAEMOGLOBIN (HB) | 15.6 | g/dL | 15.00 - 21.00 | Direct cyanmethaemoglobin method |
| PACKED CELL VOLUME (PCV) | 49.8 | % | 45.00 - 67.00 | Impedance/Calculated |
| MCV | **80.9 L** | fL | 92.00 - 118.00 | Impedance/Calculated |
| MCH | **25.4 L** | pg | 31.00 - 37.00 | |
| MCHC | 31.4 | gm/dl | 29.00 - 37.00 | Impedance/Calculated |
| RBC COUNT | 6.15 | 10^6/uL | 4.00 - 6.60 | Impedance |
| WBC COUNT | 9100 | cells/cumm | 7000.00 - 23000.00 | Impedance/Flowcytometry |
| PLATELET COUNT | 183000 | /mm3 | 160000.00 - 500000.00 | Impedance |
| **DIFFERENTIAL COUNT** | | | | |
| NEUTROPHILS | **68 H** | % | 22.00 - 42.00 | Impedance/RBC Flowcytometry |
| LYMPHOCYTES | **24 L** | % | 28.00 - 35.00 | Impedance/RBC Flowcytometry |
| EOSINOPHILS | 03 | % | 1.00 - 8.00 | Impedance/Flowcytometry |
| MONOCYTES | 05 | % | 4.00 - 7.00 | Impedance/Flowcytometry |
| BASOPHILS | 00 | % | < 1.00 | Impedance/Flowcytometry |
| **PERIPHERAL SMEAR** | | | | |
| RBC - PERIPHERAL SMEAR | NORMOCYTIC NORMOCHROMIC | | | Microscopic |
| WBC - PERIPHERAL SMEAR | NEUTROPHILIA | | | Microscopic |
| PLATELET - PERIPHERAL SMEAR | ADEQUATE | | | Microscopic |

--End Of Report--

**Dr. B Roshini Sumalini**
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Phone : 9053108108 Website : https://www.ankurahospitals.com email : info@ankurahospital.com

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646870 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 10:35PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## HEMATOLOGY

### BLOOD GROUPING AND RH TYPING

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| BLOOD GROUP | "B" | | | Slide Agglutination Method |
| RH TYPING | Positive | | | |

Comments / Interpretation:

- Reconfirm the Blood Group & Rh Typing along with cross matching before blood transfusion.
- Recent blood transfusion, if any, interferes with interpretation of blood grouping .
- The blood group and Rh antigen may change in the new born, hence please repeat the test after 6 months.
- Subgroups and Bombay blood group needs to be further verified.

--End Of Report--

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Phone : 9053108108 Website : https://www.ankurahospitals.com email : info@ankurahospital.com

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646874 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 28/06/2026 12:57AM |
| **Age/Gender** | 5 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## BIOCHEMISTRY

### RANDOM BLOOD GLUCOSE (RBS)

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| RANDOM BLOOD GLUCOSE (RBS) | 35 | mg/dL | Neonates: (1 day) (30-110)<br>Neonates :(> 1 day) (40-100)<br>Children: 60-150<br>Adults ; 60-140 | Hexokinase |

**Interpretation**

Normoglycemia : Random Plasma Glucose <140 mg/dl

Diabetes : Random Blood Glucose >/= 200 mg/dl

*Fasting and PP glucose estimation is advised when Random Blood Glucose level is 140-200 mg/dl

--End Of Report--

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## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646874 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 28/06/2026 12:57AM |
| **Age/Gender** | 5 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## BIOCHEMISTRY

### IONIC CALCIUM

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| IONIC CALCIUM | 5.08 | mg/dL | Children 4.8-5.52<br>Adult 4.64-5.28 | |

--End Of Report--

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Phone : 9053108108 Website : https://www.ankurahospitals.com email : info@ankurahospital.com

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646873 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 11:42PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 1:18AM |
| **Bed No/Ward** | IPD | **Report Date** | 28/06/2026 3:03AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## BIOCHEMISTRY

### LIVER FUNCTION TEST (LFT)

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| TOTAL BILIRUBIN | 15.5 | mg/dL | Less than 24 Hours - Below 6 mg/dl<br>24-48 Hours - Below 10 mg/dl<br>48-72 Hours - Below 12 md/dl<br>Older than 72 Hours - Below 15 mg/dl | Colorimetric diazo method |
| DIRECT BILIRUBIN | **1.8 H** | mg/dL | 0.33 - 0.71 | Diazonium |
| INDIRECT BILIRUBIN | **13.70 H** | mg/dL | 0.00 - 12.00 | |
| ALANINE TRANSMINASE (ALT/SGPT) | 10 | U/L | 10.00 - 50.00 | IFCC |
| ASPARTATE TRANSFERASE (SGOT / AST) | 35 | U/L | 10.00 - 50.00 | IFCC |
| ALKALINE PHOSPHATASE (ALP) | 214 | U/L | 83.00 - 248.00 | Colorimetric Assay |
| TOTAL PROTEIN | 5.2 | mg/dL | <15 | Turbidimetric |
| Albumin | 3.4 | g/dL | 2.8 - 5.2 | |
| Globulin | **1.8 L** | g/dL | 2.1 - 3.5 | |
| A/G Ratio | 1.9 | - | | |

Liver function tests help diagnose liver disease, determine its severity, and identify its type. These tests are typically done together, and abnormal results can indicate hepatic etiology. If multiple tests are abnormal at the same time, it's likely due to a liver problem, but if only one test is abnormal, it may be due to some other medical condion which shall be correlated clinically.

--End Of Report--

**Dr. B Roshini Sumalini**
**MBBS,MD PATHOLOGY**
**Consultant Pathologist**

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646891 |
| **UHID/IP No** | 162608635 / LB26/3007 | **Sample Date** | 28/06/2026 9:07AM |
| **Age/Gender** | 5 Days/Male | **Receiving Date** | 28/06/2026 9:08AM |
| **Bed No/Ward** | NICU Ward | **Report Date** | 28/06/2026 9:51AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## BIOCHEMISTRY

### AMMONIA

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| Ammonia | **86.0 H** | umol/L | 9.0 - 47.0 | |

--End Of Report--



**Dr. B Roshini Sumalini**
**MBBS,MD PATHOLOGY**
**Consultant Pathologist**

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of VAGADAVI | **Lab No** | 1646879 |
| **UHID/IP No** | 162608635 / LB26/3007 | **Sample Date** | 28/06/2026 6:23AM |
| **Age/Gender** | 5 Days/Male | **Receiving Date** | 28/06/2026 6:52AM |
| **Bed No/Ward** | NICU Ward | **Report Date** | 28/06/2026 10:10AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## CLINICAL PATHOLOGY

### COMPLETE URINE EXAMINATION (CUE )

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| Quantity | 20 ML | | | |
| Colour | Yellow | | | Physical |
| Appearance | Turbid | | | Physical |
| Specific gravity | 1.015 | - | 0.000 - 1.035 | PH Indicators |
| Potential of Hydrogen (PH) | 6.0 | | | Reagent Strip Method |
| Glucose | Nil | | | Hexokinase |
| Protein | + | | | |
| Bilirubin | Positive + | | | Reagent Strip method-DIa screen 50 |
| Ketone | Nil | | | Rotheras |
| BLOOD | Nil | | Absent | Physical |
| Leucocytes | Trace | | | Leucocyte Esterase |
| Urobilinogen | Negative | | | Fast Blue Color |
| Nitrate | Negative | | | |
| Pus cells | 6 - 8 | /hpf | 01-05 /hpf | Light Microscopy/Automated |
| Epithelial cells | 10 - 12 | /hpf | 01-05 /hpf | Light Microscopy/Automated |
| RBC's | Nil | | Absent /hpf | Light Microscopy/Automated |
| Crystals | Nil | | | Light Microscopy/Automated |
| Casts | Nil | | | Light Microscopy/Automated |
| OTHERS | BACTERIA PRESENT | | | |

Interpretation :
A complete urinalysis is a test that examines the urine for various abnormalities. It can detect potential health issues, such as kidney disease, urinary tract infections, or excessive levels of waste products, even if symptoms are not present. Any abnormal results require further clinical evaluation to determine their significance and underlying cause.

--End Of Report--



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## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of G VAGDEVI | **Lab No** | 1646870 |
| **UHID** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 10:35PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 10:37AM |
| **Bed No/Ward** | IPD | **Report Date** | 29/06/2026 8:16AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## MICROBIOLOGY

### CULTURE AND SENSITIVITY - AUTOMATED/VITEK , BLOOD

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| SPECIMEN TYPE | BLOOD | | | Automated colorimetric microbial detection and identification by Vitek 2 compact |
| Culture Result | PRELIMINARY 1: No bacterial growth seen after 24 hours of aerobic incubation. | | | |

Note:
The BACT/ALERT system is an automated continuous monitoring system for blood cultures that detects microbial growth by measuring color change caused by production. Microbial metabolism releases , altering pH and changing the bottle sensor color from green to yellow. The system monitors this change in real time; bottles remain undisturbed during incubation and are removed after 7 days if no growth is detected.

--End Of Report--

Gayatri

**Dr. GAYATRI KORIDE**
**MBBS, MD (Microbiology)**
**Consultant Microbiologist**

## Laboratory

| | | | |
|---|---|---|---|
| **Patient Name** | Baby Of G VAGDEVI | **Lab No** | 1646870 |
| **UHID/IP No** | 162608635 / LB26/3007 | **Sample Date** | 27/06/2026 10:35PM |
| **Age/Gender** | 4 Days/Male | **Receiving Date** | 28/06/2026 10:37AM |
| **Bed No/Ward** | NICU Ward | **Report Date** | 29/06/2026 8:16AM |
| **Referred By** | Dr. Srinivas Murki | **Report Status** | Final |
| **Patient Mob** | | | |

## MICROBIOLOGY

### CULTURE AND SENSITIVITY - AUTOMATED/VITEK , BLOOD

| Test Name | Result | Unit | Biological Ref. Interval | Method |
|---|---|---|---|---|
| SPECIMEN TYPE | BLOOD | | | Automated colorimetric microbial detection and identification by Vitek 2 compact |
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Note:
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--End Of Report--

Gayatri

**Dr. GAYATRI KORIDE**
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